

ENGLISH FOR OBSTETRICIANS

Obstetrics English

Explain the uncertainty. Respect the choice. Coordinate care.

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Eight lessons. Eleven extended conversations.

INSIDE THE BOOK

192 vocabulary entries / 128 reusable phrases
220 extended-dialogue turns / 11 transfer exchanges
Word banks and explained answer keys

Upper-intermediate to advanced (B2-C1). For obstetricians, obstetric trainees, and physicians coordinating pregnancy and postpartum care.

2026 edition

Eight scenarios. Eight skills.

Eight maternity encounters practice sensitive history, probability language, urgent escalation, nonjudgmental explanations, renewed consent, source-aware handoffs, postpartum follow-up, and difficult news.

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The plan can change; the person remains central.

Obstetric communication moves between routine planning and consequential uncertainty. Explain what is known, what a test can establish, who has a decision to make, and how the patient remains involved as circumstances change.

Do not assume the history

Ask respectfully about prior pregnancies, losses, dates, language needs, and support people. A companion is not automatically a decision-maker or an authorized recipient.

"How would you like us to refer to your previous pregnancy, and who may join this discussion?"

Name what a result can establish

Screening, diagnostic testing, imaging, and clinical assessment answer different questions. Keep probabilities distinct from confirmed findings.

"This screening result changes the estimated chance; it does not itself establish the diagnosis."

Revisit consent when the situation changes

Explain the new concern and relevant options using the actual clinical assessment. Prior preferences matter but do not remove the need for an appropriate current discussion.

"The circumstances have changed. Let me explain the recommendation and hear your questions."

Carry care beyond delivery

Postpartum concerns deserve specific communication and appropriate assessment. Do not normalize a concerning change simply because the baby has been born.

"Please tell the receiving team that you recently gave birth and describe the current symptoms."

PROFESSIONAL BOUNDARIES

All people, records, figures, and encounters are fictional. This is professional English practice, not medical advice, clinical training, a treatment protocol, or certification. Use current evidence, authorized supervision where required, local law, and your actual clinical and emergency procedures. Do not use an exercise to decide that symptoms can safely wait, select treatment, or calculate a dose. Communication about privacy, consent, and professional duties depends on jurisdiction and setting. Use qualified language support and protect real patient information.

Dates, previous experiences, and preferred words

A Read the situation

Clarify dating information and previous pregnancy experiences without making assumptions.

At an initial visit, Avery is unsure of the last menstrual period and mentions a previous pregnancy loss. Dr Stone asks permission to discuss the history, records uncertain dates as uncertain, and asks which support person Avery wants involved. No estimated due date, gestational age, or pregnancy outcome is calculated in the case.

PEOPLE / Avery: Pregnant patient | Dr Stone: Obstetrician

USE THE PERSON'S WORDS

A previous loss may carry a meaning not captured by a count. Ask respectfully, avoid assumptions about family structure, and separate necessary history from curiosity.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Which date is uncertain?

- | | |
|------------------------------------|--------------------------------------|
| A The last menstrual period | B A confirmed delivery date |
| C A booked procedure | D A completed diagnostic test |

2. What previous experience is mentioned?

- | | |
|------------------------------------|---|
| A Pregnancy loss | B A confirmed current complication |
| C A completed current birth | D A known due date |

3. What is not calculated?

- | | |
|--------------------------------------|--|
| A The need to clarify dates | B Permission to discuss history |
| C Gestational age or due date | D A privacy preference |

[Answers and explanations: page 85.](#)

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

gravity

Number of pregnancies, counted under the relevant clinical convention.

IN USE document gravity

parity

Birth history summarized using the applicable clinical convention.

IN USE clarify parity

gestational age

Duration of pregnancy expressed using the relevant dating method.

IN USE explain gestational age

estimated due date

Estimated date used for pregnancy planning and dating.

IN USE verify the estimated due date

last menstrual period

Reported starting date of the most recent menstrual period.

IN USE clarify the last menstrual period

dating ultrasound

Ultrasound used to inform pregnancy dating in context.

IN USE discuss a dating ultrasound

menstrual regularity

Pattern of how consistently menstrual cycles occur.

IN USE ask about menstrual regularity

obstetric history

Account of previous pregnancies and their relevant outcomes.

IN USE obtain an obstetric history

prior pregnancy loss

Loss in an earlier pregnancy.

IN USE acknowledge a prior pregnancy loss

live birth

Birth showing signs of life under the relevant definition.

IN USE document a live birth

preterm birth

Birth before the relevant full-term threshold.

IN USE review a preterm-birth history

ectopic pregnancy

Pregnancy implanted outside the usual uterine cavity location.

IN USE clarify an ectopic-pregnancy history

Precision check

An uncertain menstrual date must not become a precise due date by assumption. A history abbreviation does not replace clarification of what actually occurred.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

prenatal record

Record of pregnancy-related care before birth.

IN USE review the prenatal record

support person

Person the patient chooses to provide support.

IN USE identify the chosen support person

preferred terminology

Words the person wishes used for sensitive experiences.

IN USE ask about preferred terminology

trauma-informed approach

Approach attentive to safety, choice, and prior distressing experiences.

IN USE use a trauma-informed approach

dating uncertainty

Unresolved information affecting pregnancy dating.

IN USE document dating uncertainty

history clarification

Follow-up questions resolving ambiguous historical information.

IN USE request history clarification

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Counting conventions and terminology differ. Confirm the actual record and local convention; do not calculate a pregnancy outcome or dating estimate from this exercise.

Phrases you can actually use

C Notice the language / 1 of 2

ASK PERMISSION

"May we discuss your previous pregnancy experiences?"

KEEP THE RECORD HONEST

"I will record the date as uncertain."

INCLUDE SUPPORT BY CHOICE

"Who would you like involved in this conversation?"

AVOID ASSUMPTIONS

"I will not infer the outcome from an abbreviation alone."

ALLOW UNCERTAINTY

"It is fine to say that you do not remember the exact date."

ASK ABOUT WORDS

"Is there a term you would prefer us to use?"

EXPLAIN RELEVANCE

"I will explain why I am asking each sensitive question."

SUMMARIZE RESPECTFULLY

"Let me check that I have understood your history correctly."

Estimated versus guaranteed

An estimated due date is a planning estimate, not a promise of the birth date.

Unsure versus approximate

Unsure identifies uncertainty; an approximate date still requires a basis in the person's account.

Chosen support person

Identifies the patient's preference without assigning decision authority to a companion.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which entry preserves the history?

- A** Due date guaranteed.
- B** Previous loss omitted for convenience.
- C** Last menstrual period uncertain.
- D** Exact date invented from memory.

2. Which opening is respectful?

- A** Every loss means the same thing.
- B** May we discuss your previous pregnancy experiences?
- C** Your companion must answer everything.
- D** We do not need your preferred words.

3. What should an abbreviation not replace?

- A** A font choice
- B** A waiting-room number
- C** A generic slogan
- D** Clarification of the actual history

4. Which role is not automatic for a companion?

- A** Chosen support person when requested
- B** Someone who may attend with permission
- C** A person whose presence can be discussed
- D** Authorized decision-maker

[Answers and explanations: page 85.](#)

Dates, previous experiences, and preferred words

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

Parity / last menstrual period / preferred terminology / estimated due date / trauma-informed approach / obstetric history / history clarification / Dating uncertainty / support person / Gravidity

- 01 **Dr Stone:** Before we discuss dates, may I ask about previous pregnancy experiences and how you would like us to refer to them?
- 02 **Avery:** Yes. I had a pregnancy loss, and I would prefer that wording rather than moving straight to a number.
- 03 **Dr Stone:** I will respect that ¹ _____. The history matters, but the experience is more than an abbreviation.
- 04 **Avery:** I am also unsure of the exact date of my last period. I do not want to give a false answer.
- 05 **Dr Stone:** We can record the ² _____ as uncertain rather than turn a guess into a precise date.
- 06 **Avery:** Does that mean the date on my calendar is not automatically the date we should use for planning?
- 07 **Dr Stone:** Correct. ³ _____ needs to remain visible while we review the relevant information.
- 08 **Avery:** A previous form used letters and numbers that I did not understand. Could those be checked with me?
- 09 **Dr Stone:** Yes. Your ⁴ _____ should reflect what actually happened, not an assumption based only on shorthand.
- 10 **Avery:** One word was gravidity. Does that refer to every pregnancy rather than just births?

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 10.

- 11 **Dr Stone:** ⁵ _____ concerns the number of pregnancies under the relevant convention; we will clarify the details respectfully.
- 12 **Avery:** And parity is a different summary, so I should not assume the two numbers mean the same thing?
- 13 **Dr Stone:** Yes. ⁶ _____ uses the applicable birth-history convention. The underlying facts still need to be clear.
- 14 **Avery:** I would like my partner involved later, but I want this first part of the conversation to remain between us.
- 15 **Dr Stone:** We can discuss the appropriate privacy arrangements and when your chosen ⁷ _____ joins the conversation.
- 16 **Avery:** Will a scan necessarily give a guaranteed day on which the baby will be born?
- 17 **Dr Stone:** No. An ⁸ _____ is an estimate, and its basis should be explained.
- 18 **Avery:** I appreciate knowing what is uncertain instead of feeling that I must remember every detail perfectly.
- 19 **Dr Stone:** A ⁹ _____ includes explaining the purpose of questions and allowing appropriate choice and pauses.
- 20 **Avery:** Then we can complete the ¹⁰ _____ with uncertain dates marked honestly and the previous loss described in my words.

ANSWERS / EXPLANATIONS: PAGE 86

1. preferred terminology | 2. last menstrual period | 3. Dating uncertainty | 4. obstetric history | 5. Gravidity | 6. Parity | 7. support person | 8. estimated due date | 9. trauma-informed approach | 10. history clarification

A record uses unfamiliar shorthand

F Say it

An outside record contains an unfamiliar pregnancy-history abbreviation. The patient has not confirmed its meaning. The clinician requests clarification before adopting it.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

confirmed / assumption / abbreviation / history

Patient: "I do not recognize that ¹ _____."

Clinician: "Its meaning is not yet ² _____."

Patient: "We need to clarify the actual ³ _____."

Clinician: "I will not adopt it by ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 86

1. abbreviation | 2. confirmed | 3. history | 4. assumption

Rehearse with a fixed script

1. Read the corrected history discussion. Preserve uncertain dates and the preferred terminology.
2. Swap roles. Explain the difference between history terms without calculating a due date.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A higher chance is not a diagnosis

A Read the situation

Explain screening uncertainty and voluntary next steps without directing a reproductive decision.

Riley receives a higher-chance prenatal screening result and thinks it proves a fetal condition. Dr Noor explains that screening estimates chance rather than establishing a diagnosis. The actual result and available diagnostic options need individualized counseling. No probability value, procedure choice, or pregnancy decision is supplied. Riley wants time to understand the information and include a chosen support person.

PEOPLE / Riley: Pregnant patient | Dr Noor: Obstetrician

EXPLAIN CHOICES WITHOUT STEERING PERSONAL VALUES

Give clear information about the purpose and limits of testing. Do not assume what the patient would do with a result or pressure a reproductive decision.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does the result describe?

- A A required pregnancy decision
- C A confirmed diagnosis

- B A higher estimated chance
- D A guaranteed outcome

2. What does Riley request?

- A Automatic testing without discussion
- C Time and a chosen support person

- B A guaranteed normal outcome
- D A decision made by the clinician alone

3. What is not supplied?

- A A screening concern
- C Riley's question

- B A need for counseling
- D A numerical probability or procedure choice

Answers and explanations: page 87.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

prenatal screening

Testing that estimates the chance of specified conditions before birth.

IN USE explain prenatal screening

diagnostic testing

Testing intended to establish or clarify a condition.

IN USE discuss diagnostic testing

cell-free DNA screening

Screening using DNA fragments in maternal blood for specified conditions.

IN USE explain cell-free DNA screening

screening probability

Estimated chance reported by a screening process.

IN USE communicate screening probability

screen-positive result

Screening finding indicating increased chance under the relevant method.

IN USE explain a screen-positive result

false-positive result

Positive screening finding not confirmed for the condition assessed.

IN USE explain a false-positive result

false-negative result

Negative screening finding despite the condition being present.

IN USE explain a false-negative result

positive predictive value

Proportion of positive results that correspond to the condition in context.

IN USE discuss positive predictive value

residual risk

Remaining chance after a result or intervention.

IN USE explain residual risk

genetic counseling

Specialist discussion of genetic information, options, and implications.

IN USE arrange genetic counseling

amniocentesis

Procedure obtaining amniotic fluid for relevant testing.

IN USE discuss amniocentesis

chorionic villus sampling

Procedure obtaining placental tissue for relevant testing.

IN USE explain chorionic villus sampling

Precision check

A higher-chance screening result is not diagnostic confirmation. A lower-chance result does not guarantee absence of every condition.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

chromosomal condition

Condition involving chromosome number or structure.

IN USE discuss a chromosomal condition

test limitation

Boundary on what a test can detect or establish.

IN USE explain test limitations

inconclusive result

Result that does not provide a definitive answer to its question.

IN USE communicate an inconclusive result

informed refusal

Decision to decline after relevant information is understood.

IN USE document informed refusal

voluntary testing

Testing chosen without coercion after appropriate discussion.

IN USE support voluntary testing

decision support

Assistance helping a person understand options and relevant values.

IN USE offer decision support

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Explain actual test performance and options in context. Do not supply an invented probability, universal procedure recommendation, or reproductive decision.

Phrases you can actually use

C Notice the language / 1 of 2

NAME THE RESULT TYPE

"This is a screening result, not a diagnosis."

AVOID A GUARANTEE

"A lower-chance result cannot rule out every condition."

ASK ABOUT INFORMATION

"What would you like explained before considering the next step?"

INCLUDE CHOSEN SUPPORT

"We can include the person you choose with your permission."

EXPLAIN THE LIMIT

"It changes the estimated chance but does not settle the question."

OFFER COUNSELING

"We can arrange a discussion of the actual result and options."

PROTECT CHOICE

"Testing decisions require an informed, voluntary discussion."

ALLOW A PAUSE

"We can pause to make sure the information is clear."

Chance versus certainty

Keeps a probability estimate distinct from a confirmed finding.

For specified conditions

Prevents one screening method from being described as testing for everything.

May choose versus must

Preserves voluntary decision-making rather than presenting a procedure as automatic.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which explanation is accurate?

- A** Screening replaces all counseling.
- B** Screening estimates chance; diagnostic testing addresses confirmation.
- C** Every positive screen proves a condition.
- D** A negative screen guarantees every outcome.

2. Which statement overextends the test?

- A** Options can be discussed.
- B** This screens for every possible condition.
- C** We need the actual test's limits.
- D** The result requires context.

3. What does positive predictive value concern?

- A** How often a positive result corresponds to the condition in context
- B** The percentage of tests ordered
- C** A guaranteed personal outcome
- D** The price of a test

4. Which next-step statement respects choice?

- A** No explanation is needed.
- B** Let us discuss the options and your questions.
- C** You must choose my preferred procedure.
- D** Your support person decides automatically.

Answers and explanations: [page 87](#).

A higher chance is not a diagnosis

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

Decision support / prenatal screening / Residual risk / inconclusive result / Amniocentesis / Positive predictive value / false-positive result / Voluntary testing / screening probability / Genetic counseling

- 01 **Riley:** The message says higher chance. I read that as proof that the baby has the condition.
- 02 **Dr Noor:** This is ¹ _____, not diagnostic confirmation. Let us explain what this particular result can and cannot establish.
- 03 **Riley:** Does that mean the result is meaningless, or that I should simply ignore it?
- 04 **Dr Noor:** No. The ² _____ informs the next discussion, but an estimate is not certainty.
- 05 **Riley:** I would like to understand how a positive result can occur when the condition is not present.
- 06 **Dr Noor:** That is a ³ _____. The actual test and your circumstances determine how the result should be interpreted.
- 07 **Riley:** And a lower-chance result would not mean every possible condition has been excluded?
- 08 **Dr Noor:** Correct. ⁴ _____ and the scope of the test still matter.
- 09 **Riley:** Someone mentioned a predictive value, but I did not understand what was being predicted.
- 10 **Dr Noor:** ⁵ _____ concerns what positive results mean in context. We should use the actual figures, not invent a number.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 19.

- 11 **Riley:** What kind of discussion would help me understand the further testing options?
- 12 **Dr Noor:** ⁶ _____ can help explain the actual result, options, limitations, and what information you want.
- 13 **Riley:** Are amniocentesis and the placental-sampling procedure simply more screening blood tests?
- 14 **Dr Noor:** No. ⁷ _____ and chorionic villus sampling are procedures used for relevant diagnostic testing, with their own discussions of benefits and risks.
- 15 **Riley:** I do not want a procedure selected before I understand what question it would answer.
- 16 **Dr Noor:** That is appropriate. ⁸ _____ should clarify the question and your preferences rather than make the decision for you.
- 17 **Riley:** Could I include my chosen support person when we discuss the information again?
- 18 **Dr Noor:** Yes, with your permission. ⁹ _____ requires a discussion you understand, with support you choose.
- 19 **Riley:** I also want to know what happens if the further result still does not answer everything.
- 20 **Dr Noor:** We should explain the possibility of an ¹⁰ _____ and other limits before you choose the next step.

ANSWERS / EXPLANATIONS: PAGE 88

1. prenatal screening | 2. screening probability | 3. false-positive result | 4. Residual risk | 5. Positive predictive value | 6. Genetic counseling | 7. Amniocentesis | 8. Decision support | 9. Voluntary testing | 10. inconclusive result

A result is inconclusive

F Say it

A prenatal test result is described as inconclusive. The clinician arranges review of the actual report and options. No diagnosis is established.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

report / options / established / inconclusive

Patient: "The result is ¹ _____."

Clinician: "We need to review the actual ² _____."

Patient: "A diagnosis is not ³ _____."

Clinician: "We will explain the available ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 88

1. inconclusive | 2. report | 3. established | 4. options

Rehearse with a fixed script

1. Read the corrected screening exchange, emphasizing chance, confirmation, and limits.
2. Swap roles. Explain the options discussion without assigning a probability or choosing a procedure.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Symptoms first, diagnosis still under review

A Read the situation

Communicate urgent symptoms and pregnancy context without delaying the actual response.

Pregnant patient Jordan reports a severe headache and visual changes. The maternity team has already activated the actual urgent assessment pathway. Dr Ahmed confirms the symptom account and receiving team while care proceeds. No blood-pressure value, laboratory result, gestational week, or diagnosis is supplied. The exercise must not be used to decide that these symptoms can safely wait.

PEOPLE / Jordan: Pregnant patient | Dr Ahmed: Obstetrician

DO NOT NORMALIZE A SERIOUS CONCERN

Avoid dismissing a marked change as simply part of pregnancy. Communicate the concern plainly and preserve the distinction between symptoms and a diagnosis under assessment.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Which symptoms are reported?

- | | |
|---|---|
| A No current concern | B Severe headache and visual changes |
| C A confirmed laboratory diagnosis | D Normal measured blood pressure |

2. What is already underway?

- | | |
|--|---------------------------------------|
| A A completed discharge | B A language-only conversation |
| C The urgent assessment pathway | D A routine review next month |

3. What is not supplied?

- | | |
|----------------------------|--|
| A Pregnancy context | B Jordan's report |
| C A receiving team | D Blood pressure or a diagnosis |

Answers and explanations: [page 89](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

maternal warning sign

Symptom or sign requiring prompt attention in pregnancy or after birth.

IN USE communicate a maternal warning sign

severe headache

Marked head pain described or assessed in context.

IN USE report a severe headache

visual disturbance

Change in vision described by the person or assessment.

IN USE clarify a visual disturbance

hypertensive disorder

Condition involving high blood pressure in the relevant clinical context.

IN USE assess a hypertensive disorder

preeclampsia

Pregnancy-related hypertensive disorder with relevant diagnostic features.

IN USE evaluate suspected preeclampsia

blood-pressure measurement

Verified reading obtained through an appropriate measurement process.

IN USE relay a blood-pressure measurement

proteinuria

Protein present in urine, interpreted in the relevant context.

IN USE interpret proteinuria

epigastric pain

Pain in the upper central abdominal region.

IN USE clarify reported epigastric pain

right-upper-quadrant pain

Pain in the upper right abdominal region.

IN USE describe right-upper-quadrant pain

urgent assessment

Prompt clinical evaluation through the appropriate pathway.

IN USE arrange urgent assessment

maternity triage

Service assessing pregnancy-related concerns and care needs.

IN USE contact maternity triage

symptom progression

How a symptom changes over time.

IN USE report symptom progression

Precision check

The symptoms require the actual urgent clinical process. They do not authorize the learner to diagnose preeclampsia or declare measurements normal.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

clinical observation

Finding obtained through actual clinical assessment.

IN USE distinguish clinical observations

escalation pathway

Approved route for raising a concern to the appropriate care team.

IN USE activate the escalation pathway

maternal status

Current clinical condition of the pregnant or postpartum patient.

IN USE communicate maternal status

fetal assessment

Clinical evaluation of fetal condition using appropriate methods.

IN USE coordinate fetal assessment

unconfirmed diagnosis

Diagnostic possibility not yet established by evidence.

IN USE preserve an unconfirmed diagnosis

receiving service

Service accepting the relevant clinical concern and care transfer.

IN USE confirm the receiving service

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

A missing blood-pressure value is unknown, not reassuring. Keep the emergency or urgent response moving while accurate information is transferred.

Phrases you can actually use

C Notice the language / 1 of 2

03

LEAD WITH SYMPTOMS

"You report a severe headache and changes in vision."

AVOID NORMALIZATION

"I will not dismiss this as simply part of pregnancy."

CLARIFY CHANGE

"Tell the team how the symptoms have changed."

NAME THE RECEIVER

"We are confirming the receiving maternity team."

STATE THE ACTION

"The urgent assessment pathway is already active."

PRESERVE UNKNOWNNS

"We do not yet have a verified value to report."

KEEP DIAGNOSIS SEPARATE

"The cause is still being assessed."

PREVENT DELAY

"Do not wait for this conversation to finish before following the urgent process."

Reports versus confirms

The patient reports symptoms; a diagnosis requires the appropriate clinical assessment.

Already active

States that necessary urgent action precedes completion of the language task.

Unknown versus normal

Absence of a supplied measurement cannot support a normal finding.

Phrases you can actually use

C Notice the language / 2 of 2

03

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which handoff opening is accurate?

- A** Confirmed preeclampsia without assessment.
- B** Routine discomfort that can wait.
- C** Severe headache and visual changes; urgent assessment is active.
- D** Normal blood pressure without a measurement.

2. Which diagnosis is not established?

- A** The existence of a symptom report
- B** Pregnancy context
- C** An active pathway
- D** Preeclampsia

3. What does missing blood pressure mean here?

- A** Assessment is unnecessary.
- B** No verified value is supplied.
- C** The value is normal.
- D** The concern is harmless.

4. What takes priority over finishing the dialogue?

- A** Completing every blank
- B** A fictional score
- C** The actual urgent response
- D** A polished sentence

Answers and explanations: [page 89](#).

Symptoms first, diagnosis still under review

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

unconfirmed diagnosis / receiving service / maternal warning sign / urgent assessment / blood-pressure measurement / escalation pathway / symptom progression / visual disturbance / Preeclampsia / hypertensive disorder

- 01 **Jordan:** I have a severe headache and my vision has changed. I am worried that this is not my usual discomfort.
- 02 **Dr Ahmed:** The ¹ _____ pathway is already active. I will confirm your report while the clinical team continues the response.
- 03 **Jordan:** I was afraid someone would say that headaches are just part of being pregnant.
- 04 **Dr Ahmed:** A ² _____ must not be dismissed that way. We need the actual assessment, not reassurance from an assumption.
- 05 **Jordan:** Should I describe what I mean by the vision change rather than choosing a medical term?
- 06 **Dr Ahmed:** Yes. Describe the ³ _____ in your own words, including how it has changed.
- 07 **Jordan:** Does mentioning these symptoms mean you have already diagnosed a blood-pressure condition?
- 08 **Dr Ahmed:** No. A ⁴ _____ is not established by the symptom words alone.
- 09 **Jordan:** I have not been told a verified blood-pressure number during this conversation.
- 10 **Dr Ahmed:** Please flag that it is not yet available. We need an actual ⁵ _____, not an assumption based on how the patient appears.

The conversation, continued

E Conversations / turns 11-20

03

Continue with the word bank on page 28.

- 11 **Jordan:** I also want the next team to know that the symptoms have become more noticeable.
- 12 **Dr Ahmed:** We will communicate that ⁶ _____ and distinguish your report from any measured findings.
- 13 **Jordan:** Someone mentioned preeclampsia as a possible concern. Is possible different from confirmed?
- 14 **Dr Ahmed:** Yes. ⁷ _____ is not confirmed here. The team evaluates the actual clinical evidence.
- 15 **Jordan:** I understand. I do not want uncertainty about the cause to mean waiting without action.
- 16 **Dr Ahmed:** It does not. The ⁸ _____ is active while the cause is being assessed.
- 17 **Jordan:** Who is receiving the update, including the fact that I am pregnant?
- 18 **Dr Ahmed:** We are confirming the ⁹ _____ and ensuring the pregnancy context and symptom report are included.
- 19 **Jordan:** Then the symptoms are taken seriously without pretending the cause has already been proven.
- 20 **Dr Ahmed:** Exactly. We preserve the ¹⁰ _____ while the real urgent response continues, with new verified information passed to the team.

ANSWERS / EXPLANATIONS: PAGE 90

1. urgent assessment | 2. maternal warning sign | 3. visual disturbance | 4. hypertensive disorder | 5. blood-pressure measurement | 6. symptom progression | 7. Preeclampsia | 8. escalation pathway | 9. receiving service | 10. unconfirmed diagnosis

A new detail arrives during assessment

F Say it

During active urgent assessment, Jordan adds that the visual change began before the headache. The team acknowledges the corrected sequence. No diagnosis is established.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

active / unconfirmed / sequence / before

Patient: "The visual change began ¹ _____ the headache."

Clinician: "I have acknowledged the corrected ² _____."

Patient: "Assessment remains ³ _____."

Clinician: "The diagnosis remains ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 90

1. before | 2. sequence | 3. active | 4. unconfirmed

Rehearse with a fixed script

1. Read the corrected urgent exchange, leading with symptoms and the action already active.
2. Swap roles. Preserve diagnostic uncertainty without weakening the need for the actual urgent response.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

The screening result is not the whole story

A Read the situation

Explain testing stages and respond to self-blame without assigning a diagnosis or diet plan.

Morgan has a glucose screening result that requires further review under the local pregnancy-care pathway. Morgan assumes this confirms gestational diabetes and proves a personal failure. Dr Vale explains that the actual testing method and results determine the next step. No numerical cutoff, diagnostic result, meal prescription, or medicine instruction is supplied.

PEOPLE / Morgan: Pregnant patient | Dr Vale: Obstetrician

REMOVE BLAME WITHOUT MAKING FALSE ASSURANCES

Acknowledge the worry and explain the process. Avoid moral language about food or implying that every outcome is under the patient's control.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What result is available?

- | | |
|---|--|
| A A completed monitoring plan | B A screening result requiring further review |
| C A supplied confirmed diagnosis | D A medicine prescription |

2. What does Morgan assume?

- | | |
|--|---|
| A The result is already explained | B No follow-up exists |
| C A dose is supplied | D The result proves personal failure |

3. What is not supplied?

- | | |
|---------------------------------|---|
| A Morgan's concern | B A local pathway |
| C A clinician discussion | D A diagnostic cutoff or treatment instruction |

Answers and explanations: page 91.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

gestational diabetes

Diabetes first diagnosed in pregnancy under the relevant clinical criteria.

IN USE discuss gestational diabetes

glucose screening

Initial testing used in a particular glucose-assessment pathway.

IN USE explain glucose screening

glucose tolerance test

Test assessing the body's response to a specified glucose load.

IN USE discuss a glucose tolerance test

fasting sample

Sample collected under the required fasting conditions.

IN USE clarify fasting-sample instructions

postprandial

Occurring after a meal.

IN USE explain postprandial measurements

insulin resistance

Reduced response of body tissues to insulin.

IN USE explain insulin resistance

diagnostic threshold

Value or criterion used in the relevant diagnostic process.

IN USE verify diagnostic thresholds

testing pathway

Sequence of assessment steps used by the relevant service.

IN USE clarify the testing pathway

capillary glucose

Glucose measured from a small peripheral blood sample.

IN USE discuss capillary glucose

glucose log

Record of glucose readings with relevant timing and context.

IN USE review a glucose log

nutrition consultation

Individualized discussion with an appropriate nutrition professional.

IN USE arrange a nutrition consultation

individualized target

Goal selected for a particular person's clinical circumstances.

IN USE explain individualized targets

Precision check

A screening result must be interpreted within the actual testing pathway. Do not turn it into a diagnosis or a moral judgment.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

monitoring schedule

Agreed timing of measurements or checks.

IN USE confirm the monitoring schedule

meal context

Relevant information about food and timing around a measurement.

IN USE document meal context

hypoglycemia

Low blood glucose in the relevant clinical context.

IN USE explain hypoglycemia

hyperglycemia

High blood glucose in the relevant clinical context.

IN USE discuss hyperglycemia

postpartum testing

Testing undertaken after birth under the appropriate follow-up plan.

IN USE arrange postpartum testing

follow-up ownership

Identification of who is responsible for the next care step.

IN USE confirm follow-up ownership

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Testing methods and criteria vary. Verify the actual instructions and results; do not infer a dose, diet, glucose target, or safe action from this exercise.

Phrases you can actually use

C Notice the language / 1 of 2

SEPARATE THE STAGES

"We need to distinguish this screening result from a diagnostic conclusion."

USE THE ACTUAL METHOD

"The next step depends on the testing pathway used here."

AVOID INVENTED TARGETS

"Any monitoring target must come from your clinical plan."

NAME SUPPORT

"We can discuss an appropriate nutrition consultation if indicated."

REMOVE MORAL JUDGMENT

"This is a clinical question, not a score for your character."

CLARIFY PREPARATION

"We will verify the instructions for the actual test."

ASK ABOUT BARRIERS

"What might make the agreed follow-up difficult to attend?"

CARRY FOLLOW-UP FORWARD

"Let us identify who will coordinate the next result discussion."

Requires review versus confirms

A result needing review does not automatically establish a diagnosis.

Individualized

Signals that a target or plan belongs to the actual person and assessment.

After a meal

Explains postprandial in plain language without supplying a measurement schedule.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which explanation is appropriate?

- A** The actual pathway determines how this result is assessed.
- B** Every screening result is a diagnosis.
- C** The result proves a moral failure.
- D** One universal cutoff applies without context.

2. What does postprandial mean?

- A** Before every sample
- B** After a meal
- C** Before pregnancy
- D** During sleep only

3. Which item must come from the actual plan?

- A** A monitoring target
- B** Morgan's stated concern
- C** The need to clarify terminology
- D** The meaning of after

4. Which response avoids blame?

- A** You should have known every result.
- B** Good people never have abnormal screens.
- C** This is a clinical question, not a judgment of your character.
- D** You failed the pregnancy.

Answers and explanations: [page 91](#).

The screening result is not the whole story

E Conversations / turns 1-10

04

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

individualized target / Postprandial / glucose tolerance test / diagnostic threshold / fasting sample / nutrition consultation / testing pathway / glucose log / follow-up ownership / glucose screening

- 01 **Morgan:** My glucose screening result needs follow-up. Does that mean I definitely have gestational diabetes and have done something wrong?
- 02 **Dr Vale:** A ¹ _____ result is not a moral judgment. We need to review the actual method and result.
- 03 **Morgan:** I thought every service used exactly the same test and that the word positive settled everything.
- 04 **Dr Vale:** The ² _____ matters. We should explain the process used here rather than assume every test works identically.
- 05 **Morgan:** Someone mentioned a tolerance test. I do not know how that differs from the first test.
- 06 **Dr Vale:** A ³ _____ assesses the response to a glucose load. We will explain the actual procedure and instructions if it is the next step.
- 07 **Morgan:** Should I decide on my own whether to fast before an appointment?
- 08 **Dr Vale:** No. If a ⁴ _____ is required, the preparation must come from the actual instructions rather than a guess.
- 09 **Morgan:** I also heard a number from a friend. Can I use that to interpret my own result?
- 10 **Dr Vale:** A ⁵ _____ belongs to the relevant method and criteria. A number without that context can mislead.

The conversation, continued

E Conversations / turns 11-20

04

Continue with the word bank on page 37.

- 11 **Morgan:** If a diagnosis is later established, does that mean every person receives the same monitoring plan?
- 12 **Dr Vale:** No. An ⁶ _____ and any monitoring instructions come from the actual clinical plan.
- 13 **Morgan:** What does postprandial mean when people discuss the timing of a reading?
- 14 **Dr Vale:** ⁷ _____ means after a meal. The precise measurement timing still needs the verified plan.
- 15 **Morgan:** If I keep readings later, would the surrounding information matter as well as the number?
- 16 **Dr Vale:** A ⁸ _____ needs the relevant timing and context, not isolated numbers without explanation.
- 17 **Morgan:** I would appreciate support that does not treat food choices as evidence that I am a bad parent.
- 18 **Dr Vale:** An appropriate ⁹ _____ can address practical needs without blame. The actual recommendation depends on the assessment.
- 19 **Morgan:** Then the immediate task is to understand the next test and who will discuss its result with me.
- 20 **Dr Vale:** Exactly. We will confirm ¹⁰ _____ and the actual arrangements without inventing a diagnosis, target, or treatment today.

ANSWERS / EXPLANATIONS: PAGE 92

1. glucose screening | 2. testing pathway | 3. glucose tolerance test | 4. fasting sample | 5. diagnostic threshold
| 6. individualized target | 7. Postprandial | 8. glucose log | 9. nutrition consultation | 10. follow-up ownership

Preparation instructions conflict

F Say it

Two appointment messages give conflicting preparation instructions. The clinic asks the testing service to clarify before the patient follows either version.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

guessing / preparation / clarify / conflict

Patient: "The messages ¹ _____."

Clinician: "We need the testing service to ² _____."

Patient: "I should not choose a version by ³ _____."

Clinician: "We will confirm the actual ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 92

1. conflict | 2. clarify | 3. guessing | 4. preparation

Rehearse with a fixed script

1. Read the corrected exchange, separating screening, diagnosis, and individualized monitoring.
2. Swap roles. Respond to self-blame without supplying a diet, cutoff, or medicine instruction.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A changed recommendation needs a new explanation

A Read the situation

Explain a changed clinical recommendation while preserving the patient as an active decision-maker.

During care, the team recommends discussing assisted birth because the clinical circumstances have changed. Patient Quinn had hoped to avoid intervention. Dr Lane explains that the actual assessment, urgency, options, material risks, benefits, and alternatives must be discussed. The exercise supplies no fetal tracing, procedure selection, or instruction to accept or refuse a particular intervention.

PEOPLE / Quinn: Patient in maternity care | Dr Lane: Obstetrician

PREFERENCES ARE NOT A CONTRACT FOR A FIXED OUTCOME

Acknowledge the original preference without using it against the patient. Explain the changed circumstances clearly and avoid presenting a signature as the whole consent process.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What has changed?

- | | |
|---|---------------------------------|
| A A supplied procedure choice | B A guaranteed outcome |
| C The clinical circumstances and recommendation for discussion | D Quinn's legal identity |

2. What was Quinn's preference?

- | | |
|--|---|
| A Have no explanation | B Let a companion decide automatically |
| C Accept every procedure in advance | D Avoid intervention if possible |

3. What is not supplied?

- | | |
|-----------------------------------|---|
| A A need for explanation | B Quinn's concern |
| C A changed recommendation | D A procedure selection or fetal tracing |

Answers and explanations: [page 93](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

birth preference

Stated wish about care and experience during birth.

IN USE acknowledge a birth preference

assisted vaginal birth

Vaginal birth supported by an appropriate instrument under clinical indications.

IN USE discuss assisted vaginal birth

cesarean birth

Birth through an operative abdominal and uterine incision.

IN USE explain cesarean birth

clinical indication

Reason supporting a proposed intervention in the actual case.

IN USE explain the clinical indication

material risk

Risk important to the person's informed decision in context.

IN USE discuss material risks

reasonable alternative

Relevant available option to the proposed intervention.

IN USE explain reasonable alternatives

informed consent

Authorization following an appropriate discussion and understanding of the proposed care.

IN USE obtain informed consent

decision capacity

Ability to make the relevant decision under applicable standards.

IN USE assess decision capacity

time sensitivity

Degree to which timing affects the available decision process.

IN USE explain time sensitivity

analgesia

Measures intended to relieve pain.

IN USE discuss analgesia options

anesthesia

Measures producing loss of sensation for an appropriate procedure.

IN USE explain anesthesia arrangements

procedural benefit

Expected advantage of a proposed procedure in context.

IN USE explain procedural benefits

Precision check

A birth preference does not waive future explanation or guarantee an outcome. A consent signature does not replace a meaningful discussion.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

change in circumstances

New information or condition altering the relevant assessment.

IN USE describe a change in circumstances

consent discussion

Conversation addressing the proposed intervention and relevant choices.

IN USE document the consent discussion

patient question

Concern or request for explanation raised by the patient.

IN USE invite patient questions

withdrawal of consent

Retraction of previously given authorization in the relevant context.

IN USE clarify withdrawal of consent

shared decision-making

Process combining clinical information with the person's informed preferences.

IN USE support shared decision-making

communication support

Assistance enabling understandable and accessible discussion.

IN USE arrange communication support

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

The actual urgency and assessment determine the available process. This exercise does not select an intervention, interpret fetal monitoring, or supply a legal consent rule.

Phrases you can actually use

C Notice the language / 1 of 2

ACKNOWLEDGE THE PREFERENCE

"I remember that avoiding intervention was important to you."

EXPLAIN THE REASON

"Let us discuss the actual reason for considering this option."

INCLUDE ALTERNATIVES

"We need to discuss the relevant alternatives and their implications."

CHECK UNDERSTANDING

"Please tell me how you understand the recommendation."

NAME THE CHANGE

"The circumstances have changed, and I need to explain the recommendation."

MAKE TIME CLEAR

"I will explain how time-sensitive the decision is."

INVITE A FOCUSED QUESTION

"What needs clarification before you can understand the options?"

KEEP THE PATIENT INVOLVED

"Your questions and preferences remain part of this discussion."

Recommend versus require

A recommendation needs explanation and does not automatically remove the consent process.

Changed circumstances

Identifies why the discussion is being revisited without blaming the earlier preference.

Understand versus agree

Understanding the explanation does not necessarily mean agreeing to the intervention.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which statement preserves involvement?

- A** Your earlier plan means no further discussion.
- B** A signature answers every question.
- C** Your companion automatically decides.
- D** Your preferences still matter as we explain the changed recommendation.

2. What is not equivalent to consent?

- A** A relevant informed decision
- B** Consent under the applicable process
- C** Understanding the explanation
- D** Appropriate authorization after discussion

3. What should time sensitivity describe?

- A** The actual urgency and available decision process
- B** A tactic to prevent all questions
- C** A guaranteed outcome
- D** A reason to invent a tracing

4. Which clinical action is not authorized here?

- A** Explaining a consent discussion
- B** Selecting the birth intervention
- C** Asking a clarification question
- D** Acknowledging the preference

Answers and explanations: [page 93](#).

A changed recommendation needs a new explanation

E Conversations / turns 1-10

05

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

procedural benefit / time sensitivity / birth preference / informed consent / reasonable alternative / clinical indication / consent discussion / shared decision-making / assisted vaginal birth / analgesia

- 01 **Quinn:** I had hoped to avoid intervention. Hearing a different recommendation now makes me feel that my earlier wishes no longer matter.
- 02 **Dr Lane:** Your ¹ _____ still matters. I need to explain what has changed and hear your questions.
- 03 **Quinn:** Please start with the reason, not just the name of a procedure I have never heard before.
- 04 **Dr Lane:** We should discuss the actual ² _____ and the evidence behind the recommendation.
- 05 **Quinn:** I also need to know whether there is time to ask questions and include my chosen support person.
- 06 **Dr Lane:** I will explain the real ³ _____ and what communication support is appropriate in these circumstances.
- 07 **Quinn:** If you mention assisted birth, does that mean the choice has already been made for me?
- 08 **Dr Lane:** No. Discussing ⁴ _____ does not by itself establish consent or settle the choice.
- 09 **Quinn:** I would like to understand the benefit being sought and the important risks, not only hear that it is recommended.
- 10 **Dr Lane:** The ⁵ _____ and material risks belong in the explanation, with the relevant alternatives.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 46.

- 11 **Quinn:** Would those alternatives include what happens if I do not choose the proposed option?
- 12 **Dr Lane:** The appropriate ⁶ _____ discussion includes the available choices and their implications in your circumstances.
- 13 **Quinn:** Sometimes I nod because I have heard a sentence, not because I have agreed to everything in it.
- 14 **Dr Lane:** Hearing or understanding is not the same as ⁷ _____. We need to check the meaning and the decision separately.
- 15 **Quinn:** I also want the pain-relief arrangements explained if a procedure is being considered.
- 16 **Dr Lane:** We should explain the relevant ⁸ _____ or anesthesia arrangements through the appropriate clinical discussion.
- 17 **Quinn:** Could you summarize the recommendation again in shorter sentences before asking what I understand?
- 18 **Dr Lane:** Yes. That is part of a useful ⁹ _____, not a delay caused by asking an unreasonable question.
- 19 **Quinn:** Then my original preference is acknowledged, the changed situation is explained, and I can understand the actual options.
- 20 **Dr Lane:** That supports ¹⁰ _____ while respecting the real urgency and keeping you involved in the care discussion.

ANSWERS / EXPLANATIONS: PAGE 94

1. birth preference | 2. clinical indication | 3. time sensitivity | 4. assisted vaginal birth | 5. procedural benefit | 6. reasonable alternative | 7. informed consent | 8. analgesia | 9. consent discussion | 10. shared decision-making

A nod was only acknowledgment

F Say it

A patient clarifies that a nod meant the explanation was heard, not that consent was given. The clinician acknowledges the distinction and continues the actual consent process.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

heard / consent / process / question

Patient: "My nod meant I ¹ _____ the explanation."

Clinician: "It did not establish ² _____."

Patient: "I still have a ³ _____."

Clinician: "We will continue the appropriate ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 94

1. heard | 2. consent | 3. question | 4. process

Rehearse with a fixed script

1. Read the corrected exchange, distinguishing a recommendation, understanding, and consent.
2. Swap roles. Acknowledge the preference without selecting a procedure or inventing urgency.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Separate the patient report from the monitor interpretation

A Read the situation

Transfer maternal and fetal information with sources, timing, outstanding tasks, and accepted responsibility.

06

Dr Chen hands over patient Samira to Dr Ellis on the labor ward. Samira reports stronger contractions and asks that explanations precede examinations. The latest maternal observations and fetal-monitoring interpretation must be verified from the current clinical record; no values or trace classification are supplied here. The clinicians confirm outstanding tasks and responsibility without inventing findings.

PEOPLE / Dr Chen: Outgoing obstetrician | Dr Ellis: Receiving obstetrician

AN EFFICIENT HANDOFF STILL INCLUDES THE PATIENT

Include clinically relevant preferences as well as observations. Do not let a short report erase consent needs, source attribution, or unfinished work.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does Samira report?

- A A verified trace category
- C A completed birth

- B A supplied cervical measurement
- D Stronger contractions

2. What preference should be handed over?

- A Automatic consent to every procedure
- C Explain before examinations

- B A companion makes every decision
- D No questions ever

3. What requires verification?

- A That a handoff occurs
- C Current observations and monitoring interpretation

- B The receiving clinician's role
- D Samira's name in the brief

Answers and explanations: page 95.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

labor progress

Change in the clinical course of labor over time.

IN USE describe labor progress

contraction pattern

Frequency, duration, and character of uterine contractions in context.

IN USE report the contraction pattern

cervical dilation

Opening of the cervix assessed during the relevant examination.

IN USE document cervical dilation

cervical effacement

Thinning of the cervix in the relevant clinical assessment.

IN USE describe cervical effacement

fetal station

Position of the presenting part relative to pelvic landmarks.

IN USE report fetal station

membrane status

Whether membranes are intact or ruptured as assessed.

IN USE clarify membrane status

rupture time

Reported or established time of membrane rupture.

IN USE verify rupture time

presenting part

Fetal part positioned toward the birth canal.

IN USE identify the presenting part

fetal heart-rate trace

Recorded fetal heart-rate information for clinical interpretation.

IN USE review the fetal heart-rate trace

baseline rate

Reference rate assessed over the relevant monitoring interval.

IN USE describe the baseline rate

variability

Fluctuation in fetal heart rate assessed within the monitoring context.

IN USE assess variability

deceleration

Decrease in fetal heart rate characterized in clinical context.

IN USE describe a deceleration

Precision check

Patient-reported contraction strength is not a fetal-monitoring interpretation. Use verified sources and timestamps for clinical findings.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

uterine activity

Pattern of uterine contractions or related activity.

IN USE assess uterine activity

maternal observations

Verified clinical measurements or findings concerning the patient.

IN USE relay maternal observations

working assessment

Current clinical interpretation subject to relevant review.

IN USE state the working assessment

outstanding task

Required action not yet completed.

IN USE identify outstanding tasks

handoff acceptance

Explicit acknowledgment of receiving information and responsibility.

IN USE confirm handoff acceptance

contingency plan

Agreed response to specified possible changes under the actual care plan.

IN USE clarify the contingency plan

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

06

Precision check

The terminology bank is not a tracing algorithm or examination guide. No values, classification, or treatment response may be invented from this exercise.

Phrases you can actually use

C Notice the language / 1 of 2

LEAD WITH CONTEXT

"I am handing over Samira on the labor ward."

PRESERVE THE PREFERENCE

"She asks for an explanation before each examination."

SEPARATE INTERPRETATION

"The monitoring interpretation needs the actual reviewed trace."

CONFIRM RESPONSIBILITY

"Please confirm who is taking responsibility for that step."

ATTRIBUTE THE REPORT

"Samira reports stronger contractions."

VERIFY THE RECORD

"Let us check the current observations and their times."

NAME UNFINISHED WORK

"This task remains outstanding."

CLOSE THE TRANSFER

"I acknowledge the handoff and the unresolved items."

06

Reports versus assessed

Separates the patient's experience from a professional measurement or interpretation.

Current and timed

Prevents an earlier finding from being repeated as though it were current.

Accepted versus sent

Makes responsibility explicit rather than treating a transmitted report as a completed handoff.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

06

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which statement is source-aware?

- A** Dilation is invented from the report.
- B** All observations are current without checking.
- C** Samira reports stronger contractions.
- D** The trace is normal without review.

2. What does handoff acceptance add?

- A** A new diagnosis by itself
- B** A guarantee of outcome
- C** Permission to ignore preferences
- D** Confirmation of received information and responsibility

3. Which finding cannot be invented?

- A** A fetal trace classification
- B** The reported contraction change
- C** The request for explanation
- D** The existence of a receiving clinician

4. Why include the examination preference?

- A** It removes the patient's involvement.
- B** It remains relevant to communication and consent.
- C** It replaces all clinical assessment.
- D** It guarantees no examination is needed.

Answers and explanations: [page 95](#).

Separate the patient report from the monitor interpretation

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

06

WORD + PHRASE BANK

handoff acceptance / outstanding task / working assessment / Membrane status / contingency plan / Variability / fetal heart-rate trace / contraction pattern / cervical dilation / maternal observations

- 01 **Dr Chen:** I am handing over Samira. She reports stronger contractions and asks for explanations before examinations; please retain that preference.
- 02 **Dr Ellis:** I acknowledge the report. Let us distinguish the patient account from the current verified observations and clinical interpretation.
- 03 **Dr Chen:** The reported change in the ¹ _____ is her account, not a substitute for the actual assessment.
- 04 **Dr Ellis:** Understood. We should verify current findings and their times rather than copy an earlier summary as current.
- 05 **Dr Chen:** Let us review the latest ² _____ in the current record together, including when each was taken.
- 06 **Dr Ellis:** Please also identify which assessment belongs to which time, particularly where the situation has changed.
- 07 **Dr Chen:** Any ³ _____ finding needs its actual examination source and time, not an inference from stronger contractions.
- 08 **Dr Ellis:** The same applies to membrane information: reported history and established findings should not be blended.
- 09 **Dr Chen:** Yes. ⁴ _____ and any rupture time require accurate attribution.
- 10 **Dr Ellis:** What about the fetal monitoring? I need the actual reviewed interpretation rather than a reassuring shorthand label.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 55.

06

- 11 **Dr Chen:** We need the current ⁵ _____ and the authorized interpretation. No classification should be guessed from this conversation.
- 12 **Dr Ellis:** Terms such as variability have specific clinical meaning, so a language definition is not an interpretation rule.
- 13 **Dr Chen:** Correct. ⁶ _____ must be assessed in the real context, not assigned because the word appears in a handoff.
- 14 **Dr Ellis:** Let us identify which tasks are complete and which person is responsible for anything unfinished.
- 15 **Dr Chen:** Each ⁷ _____ should remain visible, including the responsible clinician and relevant communication route.
- 16 **Dr Ellis:** I will confirm responsibility rather than simply say that the message has arrived.
- 17 **Dr Chen:** That gives us ⁸ _____. Samira's request for explanation remains part of the plan.
- 18 **Dr Ellis:** We also need any actual contingency arrangements explained, not an improvised response to a trace we have not reviewed.
- 19 **Dr Chen:** Agreed. The ⁹ _____ must come from the current clinical assessment and agreed care.
- 20 **Dr Ellis:** I accept the handoff, with the ¹⁰ _____, sources, unresolved items, and patient preference clearly distinguished.

ANSWERS / EXPLANATIONS: PAGE 96

1. contraction pattern | 2. maternal observations | 3. cervical dilation | 4. Membrane status | 5. fetal heart-rate trace | 6. Variability | 7. outstanding task | 8. handoff acceptance | 9. contingency plan | 10. working assessment

An earlier observation was copied forward

F Say it

A handoff repeats an observation from 10:00 as current at 12:00. The receiving clinician identifies the timestamp error and requests the actual current record.

06

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

12:00 / 10:00 / current / timestamp

Receiver: "That observation was recorded at ¹ _____."

Sender: "The handoff is occurring at ² _____."

Receiver: "We need the actual ³ _____ record."

Sender: "I will correct the ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 96

1. 10:00 | 2. 12:00 | 3. current | 4. timestamp

Rehearse with a fixed script

1. Read the corrected handoff. Attribute the patient report separately from verified clinical findings.
2. Swap roles. Confirm responsibility without inventing a tracing category or examination value.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

The birth date belongs in the urgent handoff

A Read the situation

Communicate recent pregnancy context and urgent symptoms without normalizing them as recovery.

Twelve days after giving birth, patient Sloane reports shortness of breath and chest pain. The actual emergency response is already active. Dr Patel confirms the recent birth context and symptoms for the receiving team. No diagnosis or measured observations are supplied. This is language practice during an active response, not a guide for deciding whether postpartum symptoms can wait.

PEOPLE / Sloane: Postpartum patient | Dr Patel: Obstetrician

07

RECOVERY IS NOT AN EXPLANATION FOR EVERY SYMPTOM

Do not dismiss a concern because the patient is tired or recently gave birth. The receiving service needs both current symptoms and relevant recent pregnancy history.

Briefing checks

Circle one answer in each item. Use the briefing.

1. When did Sloane give birth?

A No birth is mentioned

C Twelve years ago

B Twelve days ago

D Today

2. Which symptoms are reported?

A No current symptoms

C A confirmed diagnosis

B Shortness of breath and chest pain

D Normal oxygen measurements

3. What is already active?

A A routine annual review

C A language assessment only

B A completed discharge from this event

D The emergency response

[Answers and explanations: page 97.](#)

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

postpartum period

Time after childbirth considered in the relevant care context.

IN USE identify the postpartum period

recent birth history

Relevant information about a birth that occurred recently.

IN USE communicate recent birth history

postpartum warning sign

Symptom or sign after birth requiring prompt attention.

IN USE report a postpartum warning sign

shortness of breath

Subjective difficulty or discomfort with breathing.

IN USE describe shortness of breath

chest pain

Pain or discomfort in the chest requiring contextual assessment.

IN USE report chest pain

thromboembolism

Obstruction caused by a blood clot traveling through circulation.

IN USE assess suspected thromboembolism

postpartum cardiomyopathy

Heart-muscle disorder arising around childbirth in the relevant clinical context.

IN USE evaluate postpartum cardiomyopathy

postpartum hemorrhage

Excessive bleeding after birth under the relevant clinical criteria.

IN USE assess postpartum hemorrhage

postpartum hypertension

High blood pressure occurring after birth in context.

IN USE evaluate postpartum hypertension

perinatal mental health

Mental health during pregnancy and the period around childbirth.

IN USE discuss perinatal mental health

intrusive thought

Unwanted thought experienced as distressing or inconsistent with wishes.

IN USE clarify an intrusive thought

suicidal thought

Thought involving ending one's own life.

IN USE ask directly about suicidal thoughts

Precision check

Shortness of breath and chest pain after birth require the actual urgent response. The terminology bank lists possible clinical concepts, not diagnoses established for Sloane.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

functional change

Difference in ability to carry out ordinary activities.

IN USE describe functional change

urgent-care route

Actual service or process for prompt clinical attention.

IN USE confirm the urgent-care route

postpartum follow-up

Planned review of health and care needs after birth.

IN USE coordinate postpartum follow-up

handover context

Relevant background accompanying current clinical information.

IN USE preserve handover context

support network

People and services providing practical or emotional help.

IN USE identify the support network

continuity of care

Connected care across clinicians, services, and time.

IN USE maintain continuity of care

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Mental-health concerns also require appropriate assessment. An intrusive thought and an intention to act are not interchangeable; do not infer safety or risk classification from a glossary definition.

Phrases you can actually use

C Notice the language / 1 of 2

STATE RECENT PREGNANCY

"I gave birth twelve days ago."

KEEP THE CONTEXT

"Please include the recent birth in the handoff."

PRESERVE DIAGNOSTIC UNCERTAINTY

"The cause has not yet been established."

SEPARATE ROUTINE FOLLOW-UP

"A scheduled postpartum visit does not replace urgent assessment."

LEAD WITH THE SYMPTOMS

"I am reporting shortness of breath and chest pain."

AVOID DISMISSAL

"We will not assume this is ordinary recovery."

CONFIRM URGENT ACTION

"The emergency response is already active."

CLOSE THE LOOP

"We will confirm that the receiving team has the relevant history."

07

After birth

Adds clinically relevant context without asserting that childbirth caused the symptom.

Not ordinary recovery by assumption

Rejects dismissal while leaving the actual diagnosis to assessment.

Routine versus urgent

Keeps a scheduled follow-up distinct from immediate clinical needs.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which detail belongs near the start of the handoff?

- A** A diagnosis chosen from the glossary
- B** Birth twelve days ago with current chest pain and breathlessness
- C** Only the next routine appointment
- D** An invented normal oxygen value

2. Which conclusion is unsupported?

- A** The cause needs assessment.
- B** Recent birth is relevant.
- C** Emergency response is active.
- D** The cause is postpartum cardiomyopathy.

3. What does a routine appointment not replace?

- A** Actual urgent assessment
- B** A calendar reminder
- C** The patient's name
- D** A document title

4. Which mental-health distinction is important?

- A** Every thought is harmless without assessment.
- B** A glossary establishes safety.
- C** An intrusive thought is not automatically an intention to act.
- D** Every unwanted thought proves intent.

[Answers and explanations: page 97.](#)

The birth date belongs in the urgent handoff

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

thromboembolism / recent birth history / postpartum period / shortness of breath / continuity of care / chest pain / Postpartum cardiomyopathy / handover context / postpartum warning sign / Postpartum follow-up

07

- 01 **Sloane:** I gave birth twelve days ago, and now I have shortness of breath and chest pain. The emergency response is active.
- 02 **Dr Patel:** I will make sure the ¹ _____ is included with those symptoms as the actual response continues.
- 03 **Sloane:** I worried that saying I was tired after birth would make people think this was ordinary recovery.
- 04 **Dr Patel:** A ² _____ should not be dismissed because you recently gave birth or feel tired.
- 05 **Sloane:** I can describe how the breathing feels, but I do not know what diagnosis explains it.
- 06 **Dr Patel:** Describe the ³ _____ in your own words. You do not need to choose a diagnosis before being heard.
- 07 **Sloane:** The chest discomfort is part of the same concern, and I want it passed on too.
- 08 **Dr Patel:** I will include the ⁴ _____ clearly rather than reduce the handoff to a general statement that you feel unwell.
- 09 **Sloane:** Does mentioning the postpartum period prove that the birth caused whatever is happening?
- 10 **Dr Patel:** No. The ⁵ _____ is relevant context, not proof of a particular cause.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 64.

- 11 **Sloane:** I have heard several frightening medical names. Should I tell the team that I definitely have one of them?
- 12 **Dr Patel:** No. For example, ⁶ _____ is not a diagnosis you should assign from a list of terms.
- 13 **Sloane:** And the same is true of the heart condition people sometimes mention around childbirth?
- 14 **Dr Patel:** Yes. ⁷ _____ requires the actual clinical evaluation; it has not been established in this conversation.
- 15 **Sloane:** I have a routine follow-up booked, but I understand that does not replace what is happening now.
- 16 **Dr Patel:** Correct. ⁸ _____ and the active urgent response are different parts of care.
- 17 **Sloane:** Please confirm that the next team knows the timing of the birth, not only the symptoms.
- 18 **Dr Patel:** We will preserve that ⁹ _____ and confirm receipt through the actual clinical process.
- 19 **Sloane:** I want the teams to stay connected afterward as well, rather than assume another service has everything.
- 20 **Dr Patel:** That is part of ¹⁰ _____. The immediate response remains the priority, with appropriate communication between services as care proceeds.

07

ANSWERS / EXPLANATIONS: PAGE 98

1. recent birth history | 2. postpartum warning sign | 3. shortness of breath | 4. chest pain | 5. postpartum period | 6. thromboembolism | 7. Postpartum cardiomyopathy | 8. Postpartum follow-up | 9. handover context | 10. continuity of care

Recent pregnancy was missing from the summary

F Say it

An urgent handoff lists current symptoms but omits that the patient gave birth twelve days ago. The receiving team acknowledges the added history. The response remains active.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

active / twelve / established / history

Sender: "The omitted detail is birth ¹ _____ days ago."

Receiver: "I acknowledge the added ² _____."

Sender: "The response remains ³ _____."

Receiver: "The cause is not yet ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 98

1. twelve | 2. history | 3. active | 4. established

Rehearse with a fixed script

1. Read the corrected exchange. Put the birth timing and current symptoms at the start.
2. Swap roles. Distinguish urgent response from routine follow-up without naming a confirmed cause.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Clear words and space to respond

A Read the situation

Give difficult news directly and compassionately, then explain an individualized options discussion.

After the appropriate clinical assessment has confirmed an early pregnancy loss, Dr Gray meets patient Eden. Eden asks whether the finding is certain and worries that an ordinary daily activity caused the loss. The cause has not been established. Dr Gray explains the confirmed finding, allows silence, asks about chosen support, and arranges discussion of the actual management options. No procedure, medicine, or follow-up interval is prescribed.

PEOPLE / Dr Gray: Obstetrician | Eden: Patient

08

DO NOT RUSH TOWARD REASSURANCE

Avoid minimizing phrases such as at least it was early or you can try again. State the confirmed finding clearly, allow a response, and ask what support the person wants.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What has been confirmed?

- | | |
|-------------------------------------|---|
| A A future pregnancy outcome | B An early pregnancy loss after appropriate assessment |
| C The cause of the loss | D A selected management option |

2. What worries Eden?

- | | |
|--|---|
| A That no assessment occurred | B That the future is guaranteed |
| C That an ordinary activity caused the loss | D That a procedure is already supplied |

3. What is not prescribed?

- | | |
|--|------------------------------------|
| A Space to respond | B A support discussion |
| C A management option or follow-up interval | D Compassionate explanation |

Answers and explanations: page 99.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

early pregnancy loss

Loss of a pregnancy early in gestation under the relevant definition.

IN USE explain early pregnancy loss

miscarriage

Common term for spontaneous pregnancy loss in the relevant clinical context.

IN USE discuss a miscarriage

confirmed finding

Conclusion established through the appropriate assessment.

IN USE explain a confirmed finding

diagnostic uncertainty

Unresolved question about whether a diagnosis is established.

IN USE distinguish diagnostic uncertainty

viability assessment

Clinical assessment of whether a pregnancy is continuing as expected.

IN USE review the viability assessment

expectant management

Management involving observation under an appropriate clinical plan.

IN USE discuss expectant management

medical management

Management using prescribed medicines under the actual care plan.

IN USE explain medical management

surgical management

Management involving an appropriate procedure.

IN USE discuss surgical management

management preference

Person's informed preference among relevant care options.

IN USE elicit a management preference

retained pregnancy tissue

Pregnancy-related tissue remaining in the uterus in context.

IN USE assess retained pregnancy tissue

bleeding history

Account of bleeding timing, amount, and associated concerns.

IN USE obtain a bleeding history

follow-up assessment

Subsequent review of clinical status and care needs.

IN USE arrange follow-up assessment

Precision check

Here the loss is confirmed; its cause is not. Do not weaken a confirmed finding with misleading ambiguity or invent a cause to answer self-blame.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

bereavement support

Support offered after a significant loss.

IN USE offer bereavement support

self-blame

Belief that one is personally responsible for an adverse event.

IN USE respond to self-blame

empathetic silence

Deliberate space allowing a person to respond to difficult news.

IN USE allow empathetic silence

acknowledgment of loss

Direct recognition of the significance of the person's loss.

IN USE offer acknowledgment of loss

support preference

Person's wishes about who or what will support them.

IN USE ask about support preferences

care summary

Clear record of the finding, discussion, and agreed next steps.

IN USE provide a care summary

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Management depends on the actual clinical situation and informed discussion. The vocabulary names options without supplying a regimen, procedure choice, or safe waiting period.

Phrases you can actually use

C Notice the language / 1 of 2

GIVE CLEAR NEWS

"I am sorry. The assessment has confirmed that the pregnancy has ended."

SEPARATE CERTAINTY

"The finding is confirmed; the cause has not been established."

AVOID MINIMIZATION

"I recognize that this is a significant loss for you."

INTRODUCE OPTIONS GENTLY

"When you are ready, I will explain the options relevant to your situation."

ALLOW SPACE

"We can pause here."

RESPOND TO BLAME

"Nothing in this assessment establishes that your ordinary activity caused this."

ASK ABOUT SUPPORT

"Who would you like with you for the next discussion?"

CLOSE CONCRETELY

"We will provide the agreed care plan and the appropriate contact route."

08

Confirmed versus suspected

Matches the certainty of the actual finding instead of using vague words to soften difficult news.

Cause versus finding

Explains that certainty about what happened does not establish why it happened.

When you are ready

Makes space within the actual clinical circumstances without promising unlimited delay or prescribing timing.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which statement matches the case?

- A** Both the loss and cause are guesses.
- B** An ordinary activity definitely caused it.
- C** A future outcome is guaranteed.
- D** The loss is confirmed, but the cause is not established.

2. Which phrase minimizes the loss?

- A** Who would you like with you?
- B** At least it was early.
- C** I am sorry.
- D** We can pause here.

3. What does empathetic silence allow?

- A** A guarantee that distress ends
- B** Space for the person to respond
- C** Avoidance of every necessary explanation
- D** A substitute for clinical care

4. Which action is not supplied?

- A** Explaining relevant options
- B** Asking about support
- C** Providing an agreed summary
- D** Selecting a management regimen

Answers and explanations: page 99.

Clear words and space to respond

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

confirmed finding / support preference / management preference / care summary / self-blame / expectant management / Medical management / bereavement support / Acknowledgment of loss / Empathetic silence

08

- 01 **Eden:** Please tell me clearly what the assessment has established. Is the finding definite, or is there still uncertainty?
- 02 **Dr Gray:** I am sorry. The pregnancy has ended. This is a ¹ _____. We can pause before discussing anything further.
- 03 **Eden:** I keep thinking about carrying groceries yesterday. Did something ordinary I did cause this?
- 04 **Dr Gray:** The assessment does not establish that cause. I hear the ² _____, but we should not turn an ordinary activity into a proven explanation.
- 05 **Eden:** I do not want someone to tell me it does not matter because it happened early.
- 06 **Dr Gray:** It matters. ³ _____ means recognizing your experience, not comparing it with someone else's.
- 07 **Eden:** Could we sit quietly for a moment before talking about procedures or what happens next?
- 08 **Dr Gray:** Yes. ⁴ _____ can be part of this conversation. I will stay with you while we pause.
- 09 **Eden:** I would like my sister involved, but only after I have understood the main information myself.
- 10 **Dr Gray:** We will respect that ⁵ _____ and discuss what you want shared when she joins us.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 73.

- 11 **Eden:** When we talk about care, will you explain the options rather than assume I already know the medical words?
- 12 **Dr Gray:** Yes. Where appropriate, ⁶ _____ is one term we may discuss, with the actual circumstances and safety arrangements explained.
- 13 **Eden:** I have also seen references to medicines and procedures, but I do not know what applies to me.
- 14 **Dr Gray:** ⁷ _____ and surgical management need their own explanations of suitability, benefits, risks, and alternatives.
- 15 **Eden:** I would like time to understand the choices within whatever the actual clinical situation allows.
- 16 **Dr Gray:** Your ⁸ _____ belongs in that discussion. We will explain any real timing considerations rather than assume your decision.
- 17 **Eden:** I may need support after leaving, not just information during this appointment.
- 18 **Dr Gray:** We can discuss ⁹ _____ and the appropriate clinical contact route, according to what would be helpful to you.
- 19 **Eden:** A clear summary would help me remember what was confirmed and what still needs to be decided.
- 20 **Dr Gray:** We will provide a ¹⁰ _____ that separates the finding, the unresolved cause, and the next steps actually agreed with you.

08

ANSWERS / EXPLANATIONS: PAGE 100

1. confirmed finding | 2. self-blame | 3. Acknowledgment of loss | 4. Empathetic silence | 5. support preference | 6. expectant management | 7. Medical management | 8. management preference | 9. bereavement support | 10. care summary

The support person joins with permission

F Say it

Eden authorizes sharing the confirmed finding with her sister. Eden has not selected a management option. The clinician explains those two facts separately.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

selected / sister / finding / separate

Patient: "You may share the confirmed ¹ _____."

Clinician: "The support person is your ² _____."

Patient: "A management option is not yet ³ _____."

Clinician: "I will keep those facts ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 100

1. finding | 2. sister | 3. selected | 4. separate

Rehearse with a fixed script

1. Read the corrected difficult-news conversation. Allow a pause after the confirmed finding.
2. Swap roles. Respond to self-blame without inventing a cause or minimizing the loss.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A sensitive consultation with language support

E More workplace conversations / 1-10

Arrange qualified interpretation and private discussion without assigning authority to a companion.

Patient Noor asks Dr Ellis for qualified interpretation and private time without a companion before discussing sensitive maternity information. The actual interpreter is arranged through the service. This dialogue concerns preferences, privacy, and preparation; it supplies no clinical decision or jurisdiction-specific rule.

PEOPLE / Noor: Patient | Dr Ellis: Obstetrician

09

Use each bank entry once, without changing its form. Answers follow turn 20.

WORD + PHRASE BANK

qualified language support / language preference / decision-making role / clarification request / communication agreement / disclosure permission

- 01 **Noor:** I would like an interpreter, but I do not want my companion to interpret the personal questions for me.
- 02 **Dr Ellis:** We can arrange ¹ _____ through the service and discuss the appropriate privacy arrangements.
- 03 **Noor:** My companion speaks more English, but that does not mean I want every detail shared with them.
- 04 **Dr Ellis:** Correct. Language ability does not create ² _____ or decision-making authority.
- 05 **Noor:** Could I have some time alone first, before deciding what I want discussed with the companion present?
- 06 **Dr Ellis:** We can arrange an appropriate private conversation and explain any relevant privacy limits in this setting.
- 07 **Noor:** I would also like the interpreter to use the language variety I understand best.
- 08 **Dr Ellis:** We should confirm your ³ _____ rather than infer it from your name or country.
- 09 **Noor:** When the interpreter arrives, will you still speak to me directly?
- 10 **Dr Ellis:** Yes. I will address you, use manageable segments, and allow the interpretation to finish before adding another question.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 76.

- 11 **Noor:** If I do not understand a medical term, I want to be able to ask without feeling that I am delaying everything.
- 12 **Dr Ellis:** A ⁴ _____ is useful. We should explain the meaning, not ask you to agree to words you do not understand.
- 13 **Noor:** I may want my companion involved later, once I know what information is being discussed.
- 14 **Dr Ellis:** We can revisit that choice and specify the appropriate sharing rather than assume today's preference is unlimited or permanent.
- 15 **Noor:** Does the interpreter make a decision for me if I cannot express the answer fluently in English?
- 16 **Dr Ellis:** No. The interpreter supports communication. Your ⁵ _____ is not transferred because you need language support.
- 17 **Noor:** Could we check the contact method too? The telephone number in the record is shared.
- 18 **Dr Ellis:** We should verify the actual privacy of that route before sending sensitive information or promising confidentiality.
- 19 **Noor:** That helps me understand what will happen before I discuss the personal details.
- 20 **Dr Ellis:** A clear ⁶ _____ can record the relevant preferences while we follow the actual clinical and privacy processes.

09

ANSWERS / EXPLANATIONS: PAGE 101

1. qualified language support | 2. disclosure permission | 3. language preference | 4. clarification request | 5. decision-making role | 6. communication agreement

The contact number is shared

F Say it in a new situation

The listed telephone number is shared with a companion. The patient requests a different contact route, which the service has not yet verified.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

route / information / shared / verified

09

Patient: "The listed number is ¹ _____."

Clinician: "You have requested a different ² _____."

Patient: "That route is not yet ³ _____."

Clinician: "We will check before sending sensitive ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 101

1. shared | 2. route | 3. verified | 4. information

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

Discussing postpartum contraception preferences

E More workplace conversations / 1-10

Explore priorities and explain that suitability needs an individualized review without imposing a method.

Patient Alex asks Dr Vale about contraception after birth. Alex wants to discuss reversibility, bleeding concerns, feeding, convenience, and future pregnancy preferences. The clinician explains that medical eligibility, timing, and available methods require the actual individual assessment. No method, prescription, or eligibility rule is selected.

PEOPLE / Alex: Postpartum patient | Dr Vale: Obstetrician

Use each bank entry once, without changing its form. Answers follow turn 20.

10

WORD + PHRASE BANK

Reversibility / voluntary choice / Medical eligibility / bleeding concern / preference discussion / reproductive preferences

- 01 **Alex:** I want to discuss contraception, but I do not want a method selected before you know what matters to me.
- 02 **Dr Vale:** Let us start with your priorities and the actual clinical assessment, not assume one option fits everyone.
- 03 **Alex:** I may want another pregnancy later, but I do not want to decide on an exact date today.
- 04 **Dr Vale:** We can discuss ¹ _____ without pressuring you to choose a timetable you have not decided.
- 05 **Alex:** I also want to know whether a method can be stopped if my wishes or circumstances change.
- 06 **Dr Vale:** ² _____ is an important comparison, with the actual process and limitations explained for each relevant option.
- 07 **Alex:** Bleeding changes would matter to me because a previous experience was difficult.
- 08 **Dr Vale:** We should include your ³ _____ in the discussion rather than dismiss it as a minor detail.
- 09 **Alex:** Does feeding my baby automatically mean every method has the same suitability and timing?
- 10 **Dr Vale:** No. ⁴ _____ and timing require the individual assessment and current applicable guidance.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 79.

- 11 **Alex:** I need something practical, but I do not want convenience to be the only factor considered.
- 12 **Dr Vale:** We can compare the actual options across the features that matter to you, including how they are used and accessed.
- 13 **Alex:** If a family member prefers a particular method, does that decide what I should receive?
- 14 **Dr Vale:** Your ⁵ _____ remains central. Support from others should not become pressure to accept a method.
- 15 **Alex:** I would like benefits, limitations, and possible effects explained in ordinary language.
- 16 **Dr Vale:** That is appropriate. We should also explain what the method does not address, using its actual information.
- 17 **Alex:** Can we document what I want to discuss without treating it as consent to a procedure?
- 18 **Dr Vale:** Yes. A ⁶ _____ is not the same as consent to an intervention or a prescription.
- 19 **Alex:** Then I can compare the relevant options after the clinical assessment instead of agreeing to a method I do not understand.
- 20 **Dr Vale:** Exactly. We will explain the actual choices and next step without inventing an eligibility rule or deciding for you.

10

ANSWERS / EXPLANATIONS: PAGE 102

1. reproductive preferences | 2. Reversibility | 3. bleeding concern | 4. Medical eligibility | 5. voluntary choice | 6. preference discussion

A preference is recorded, no method chosen

F Say it in a new situation

The patient says reversibility matters and asks about possible bleeding changes. No method has been selected or prescribed.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

selected / Reversibility / prescribed / bleeding

10

Patient: "1 _____ matters to me."

Clinician: "You also asked about 2 _____ changes."

Patient: "A method has not been 3 _____."

Clinician: "Nothing has been 4 _____ in this scenario."

ANSWERS / EXPLANATIONS: PAGE 102

1. Reversibility | 2. bleeding | 3. selected | 4. prescribed

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

An uncertain ultrasound finding

E More workplace conversations / 1-10

Explain an unresolved finding and the purpose of specialist review without converting uncertainty into a diagnosis.

Patient Rowan receives an ultrasound report recommending specialist review of an uncertain finding. Dr Kim explains that the wording does not establish a diagnosis or guarantee that nothing is wrong. The actual report and clinical context will be reviewed. No finding measurements, prognosis, or procedure choice are supplied.

PEOPLE / Rowan: Pregnant patient | Dr Kim: Obstetrician

Use each bank entry once, without changing its form. Answers follow turn 20.

WORD + PHRASE BANK

measurement limitation / uncertain finding / specialist review / Result follow-up / further-test decision / source images

11

- 01 **Rowan:** The ultrasound report recommends another review. Does that mean a serious diagnosis has already been confirmed?
- 02 **Dr Kim:** The wording describes an ¹ _____, not a confirmed diagnosis. We should review the actual report and context carefully.
- 03 **Rowan:** I also do not want to be told that it definitely means nothing when the report asks for more assessment.
- 04 **Dr Kim:** That would be an unsupported guarantee. We need to explain what is known and what remains unresolved.
- 05 **Rowan:** What is the purpose of seeing a specialist rather than simply repeating the same reassurance?
- 06 **Dr Kim:** A ² _____ can clarify the question and appropriate next steps without predetermining the conclusion.
- 07 **Rowan:** Is the report's recommendation the same as an appointment being booked?
- 08 **Dr Kim:** No. We need to distinguish the recommendation, the referral, and an actual confirmed appointment.
- 09 **Rowan:** I would like the specialist to have the original images and not only a sentence copied into a letter.
- 10 **Dr Kim:** The relevant ³ _____ and report should be available through the authorized transfer process.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 82.

- 11 **Rowan:** If a measurement is uncertain, should I compare it with a number I found from another pregnancy?
- 12 **Dr Kim:** A ⁴ _____ needs the actual context; an unrelated number may not answer your question.
- 13 **Rowan:** I want to understand what the next assessment might clarify and what it may still leave uncertain.
- 14 **Dr Kim:** We should explain that explicitly, including any relevant options once the actual clinical review identifies them.
- 15 **Rowan:** Could I include my chosen support person when the result is discussed?
- 16 **Dr Kim:** Yes, with the appropriate permission. We can also arrange language support if needed for a clear explanation.
- 17 **Rowan:** Please do not treat my questions as agreement to a particular test before it has been explained.
- 18 **Dr Kim:** Questions support understanding. Any ⁵ _____ needs its own appropriate discussion and authorization.
- 19 **Rowan:** Then we should confirm the referral status and who will explain the specialist's findings afterward.
- 20 **Dr Kim:** Correct. ⁶ _____ should be explicit, with uncertainty preserved until the actual assessment supports a conclusion.

11

ANSWERS / EXPLANATIONS: PAGE 103

1. uncertain finding | 2. specialist review | 3. source images | 4. measurement limitation | 5. further-test decision
| 6. Result follow-up

The referral is sent but images are missing

F Say it in a new situation

The specialist service has received the referral letter but not the source images. The appointment is not confirmed. The finding remains uncertain.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

confirmed / missing / uncertain / received

Patient: "The source images are ¹ _____."

Clinician: "The referral letter has been ² _____."

Patient: "The appointment is not ³ _____."

Clinician: "The finding remains ⁴ _____."

11

ANSWERS / EXPLANATIONS: PAGE 103

1. missing | 2. received | 3. confirmed | 4. uncertain

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

01 Briefing and language checks

Taking a sensitive pregnancy history

A Briefing checks / page 4

1. Which date is uncertain?

A. The last menstrual period

Avery is unsure of the last menstrual period in the supplied history.

2. What previous experience is mentioned?

A. Pregnancy loss

Avery mentions a previous loss without supplying a detailed obstetric history.

3. What is not calculated?

C. Gestational age or due date

The case supplies no data or instruction for calculating gestational age or a due date.

D Language checks / page 9

1. Which entry preserves the history?

C. Last menstrual period uncertain.

The entry accurately retains uncertainty rather than manufacturing precision.

2. Which opening is respectful?

B. May we discuss your previous pregnancy experiences?

Asking permission creates space for a sensitive history without assuming the person's experience.

3. What should an abbreviation not replace?

D. Clarification of the actual history

Clinical shorthand may use different conventions and cannot replace the relevant facts.

4. Which role is not automatic for a companion?

D. Authorized decision-maker

Accompanying a patient does not by itself establish decision-making or information-sharing authority.

01 Conversation answers

Taking a sensitive pregnancy history

E Dialogue gaps

Pages 10-11

1. preferred terminology. Turn 3. Preferred terminology records the person's chosen words for a sensitive experience rather than imposing a label without checking its meaning.

2. last menstrual period. Turn 5. The last menstrual period is a reported dating detail and should remain uncertain when the patient cannot supply a reliable date.

3. Dating uncertainty. Turn 7. Dating uncertainty identifies unresolved information affecting pregnancy dating and should remain visible while the appropriate assessment is arranged.

4. obstetric history. Turn 9. Obstetric history describes prior pregnancies and relevant outcomes and should be clarified rather than inferred solely from unfamiliar shorthand.

5. Gravidity. Turn 11. Gravidity counts pregnancies under the relevant convention and differs from a summary of birth history.

6. Parity. Turn 13. Parity summarizes birth history using the applicable clinical convention and is not interchangeable with gravidity.

7. support person. Turn 15. A support person is someone the patient chooses for support and is not automatically entitled to every disclosure or decision.

8. estimated due date. Turn 17. An estimated due date is a planning and dating estimate rather than a guarantee of the actual day of birth.

9. trauma-informed approach. Turn 19. A trauma-informed approach attends to safety, choice, and prior distress without forcing unnecessary disclosure or ignoring the person's preferences.

10. history clarification. Turn 20. History clarification resolves ambiguous information through respectful follow-up rather than replacing missing details with assumptions.

F Transfer conversation

Page 12

1. abbreviation

Patient: "I do not recognize that abbreviation."

The outside record contains shorthand the patient does not recognize.

2. confirmed

Clinician: "Its meaning is not yet confirmed."

The patient has not confirmed what the abbreviation represents.

3. history

Patient: "We need to clarify the actual history."

The underlying pregnancy history matters more than assuming the shorthand is accurate.

4. assumption

Clinician: "I will not adopt it by assumption."

The clinician requests clarification rather than treating the unverified entry as established.

02 Briefing and language checks

Explaining prenatal screening choices

A Briefing checks / page 13

1. What does the result describe?

B. A higher estimated chance

The case describes screening probability rather than diagnostic confirmation.

2. What does Riley request?

C. Time and a chosen support person

Riley wants to understand the information with appropriate support.

3. What is not supplied?

D. A numerical probability or procedure choice

The exercise supplies neither an individual probability value nor a selected diagnostic procedure.

D Language checks / page 18

1. Which explanation is accurate?

B. Screening estimates chance; diagnostic testing addresses confirmation.

Screening and diagnosis answer different questions and cannot be treated as interchangeable.

2. Which statement overextends the test?

B. This screens for every possible condition.

A screening method has a defined scope and cannot be described as detecting every condition.

3. What does positive predictive value concern?

A. How often a positive result corresponds to the condition in context

The measure concerns the meaning of positive results in the relevant population and context.

4. Which next-step statement respects choice?

B. Let us discuss the options and your questions.

An informed discussion supports the patient's decision without imposing a reproductive choice.

02 Conversation answers

Explaining prenatal screening choices

E Dialogue gaps

Pages 19-20

- 1. prenatal screening.** Turn 2. Prenatal screening estimates chance; it does not itself establish a diagnosis.
- 2. screening probability.** Turn 4. Screening probability informs discussion but is not certainty about an individual outcome.
- 3. false-positive result.** Turn 6. A false-positive result is not confirmed for the condition being assessed.
- 4. Residual risk.** Turn 8. Residual risk remains after screening; lower chance is not a universal guarantee.
- 5. Positive predictive value.** Turn 10. Positive predictive value describes positive results in context, not a personal guarantee.
- 6. Genetic counseling.** Turn 12. Genetic counseling supports understanding of genetic information, testing options, limitations, and implications without imposing a personal reproductive decision.
- 7. Amniocentesis.** Turn 14. Amniocentesis obtains amniotic fluid for relevant testing and is not simply another screening blood draw.
- 8. Decision support.** Turn 16. Decision support helps the patient understand options and relevant values rather than choosing a procedure or pregnancy outcome for them.
- 9. Voluntary testing.** Turn 18. Voluntary testing is chosen without coercion after an appropriate informed discussion, not automatically required by a screening result.
- 10. inconclusive result.** Turn 20. An inconclusive result leaves its relevant question unresolved and should be discussed without promising that every test will provide certainty.

F Transfer conversation

Page 21

1. inconclusive

Patient: "The result is inconclusive."

The result does not provide a definitive answer to its relevant question.

2. report

Clinician: "We need to review the actual report."

The actual report is the basis for explaining the unresolved result.

3. established

Patient: "A diagnosis is not established."

An inconclusive result does not establish a diagnosis in this case.

4. options

Clinician: "We will explain the available options."

The next step is an informed review of options rather than an automatic decision.

03 Briefing and language checks

Escalating an urgent maternal concern

A Briefing checks / page 22

1. Which symptoms are reported?

B. Severe headache and visual changes

The supplied report names severe headache and visual changes.

2. What is already underway?

C. The urgent assessment pathway

The actual urgent response is already active and continues during communication.

3. What is not supplied?

D. Blood pressure or a diagnosis

No measurements or diagnostic conclusion are provided for the learner to invent.

D Language checks / page 27

1. Which handoff opening is accurate?

C. Severe headache and visual changes; urgent assessment is active.

The opening states the supplied symptoms and action without inventing a diagnosis or safe delay.

2. Which diagnosis is not established?

D. Preeclampsia

The case supplies no diagnostic evidence establishing preeclampsia.

3. What does missing blood pressure mean here?

B. No verified value is supplied.

An absent measurement is unknown and must not be converted into reassurance.

4. What takes priority over finishing the dialogue?

C. The actual urgent response

Language practice must not delay the real clinical response to the concern.

03 Conversation answers

Escalating an urgent maternal concern

E Dialogue gaps

Pages 28-29

1. urgent assessment. Turn 2. Urgent assessment is already active and must not wait for this conversation.

2. maternal warning sign. Turn 4. A maternal warning sign needs prompt attention, not dismissal as ordinary pregnancy.

3. visual disturbance. Turn 6. Visual disturbance describes the reported change and needs clarification in the patient's words.

4. hypertensive disorder. Turn 8. A hypertensive disorder requires assessment; symptom words alone do not establish it.

5. blood-pressure measurement. Turn 10. Blood-pressure measurement requires an actual verified reading, not assumed normality.

6. symptom progression. Turn 12. Symptom progression describes change over time and should be passed on accurately without converting it into an unsupported diagnostic conclusion.

7. Preeclampsia. Turn 14. Preeclampsia is a pregnancy-related hypertensive disorder requiring relevant diagnostic evidence and remains unconfirmed in this language case.

8. escalation pathway. Turn 16. The escalation pathway moves the concern to the appropriate clinical team and can be active while the precise diagnosis remains uncertain.

9. receiving service. Turn 18. The receiving service accepts the clinical concern and transfer and should be explicitly confirmed rather than assumed from a sent message.

10. unconfirmed diagnosis. Turn 20. An unconfirmed diagnosis is a possibility not yet established; keeping that uncertainty visible does not delay necessary urgent assessment.

F Transfer conversation

Page 30

1. before

Patient: "The visual change began before the headache."

The patient clarifies the symptom sequence in the supplied update.

2. sequence

Clinician: "I have acknowledged the corrected sequence."

The update changes the order of symptoms in the history.

3. active

Patient: "Assessment remains active."

The actual urgent assessment continues during the clarification.

4. unconfirmed

Clinician: "The diagnosis remains unconfirmed."

The corrected timeline does not independently establish a diagnosis.

04 Briefing and language checks

Explaining a glucose-testing pathway

04

A Briefing checks / page 31

1. What result is available?

B. A screening result requiring further review

The case gives a screening result needing review, not a diagnostic conclusion.

2. What does Morgan assume?

D. The result proves personal failure

Morgan interprets the result as both a diagnosis and a moral judgment.

3. What is not supplied?

D. A diagnostic cutoff or treatment instruction

The exercise supplies no values or treatment plan for clinical use.

D Language checks / page 36

1. Which explanation is appropriate?

A. The actual pathway determines how this result is assessed.

The testing method and relevant criteria determine the clinical interpretation.

2. What does postprandial mean?

B. After a meal

Postprandial identifies a relationship to a meal rather than a universal testing instruction.

3. Which item must come from the actual plan?

A. A monitoring target

Targets depend on the individual clinical plan and are not provided by this exercise.

4. Which response avoids blame?

C. This is a clinical question, not a judgment of your character.

The response removes moral judgment while preserving the need for appropriate clinical review.

04 Conversation answers

Explaining a glucose-testing pathway

E Dialogue gaps

Pages 37-38

1. glucose screening. Turn 2. Glucose screening is an assessment stage, not automatically a completed diagnosis.

2. testing pathway. Turn 4. The testing pathway determines how this screening result leads to further assessment.

3. glucose tolerance test. Turn 6. A glucose tolerance test needs the actual procedure and preparation explained.

4. fasting sample. Turn 8. Fasting-sample preparation must come from verified instructions rather than a guess.

5. diagnostic threshold. Turn 10. A diagnostic threshold belongs to a particular method and clinical process and cannot be borrowed without checking that it applies.

6. individualized target. Turn 12. An individualized target is chosen for the person's clinical circumstances and should not be invented from another person's plan.

7. Postprandial. Turn 14. Postprandial means after a meal and explains timing terminology without prescribing a universal measurement schedule.

8. glucose log. Turn 16. A glucose log records readings with relevant timing and context so they can be interpreted under the actual clinical plan.

9. nutrition consultation. Turn 18. A nutrition consultation provides individualized professional discussion rather than moral judgment or a generic diet prescribed by this language case.

10. follow-up ownership. Turn 20. Follow-up ownership identifies the professional or service responsible for the next step so a result does not remain unexplained.

F Transfer conversation

Page 39

1. conflict

Patient: "The messages conflict."

The two appointment messages contain inconsistent instructions for test preparation.

2. clarify

Clinician: "We need the testing service to clarify."

The responsible testing service must resolve the conflicting instructions.

3. guessing

Patient: "I should not choose a version by guessing."

The scenario calls for clarification rather than guessing which version applies.

4. preparation

Clinician: "We will confirm the actual preparation."

The verified preparation for the actual test is the required information.

05 Briefing and language checks

Revisiting a birth preference

A Briefing checks / page 40

1. What has changed?

C. The clinical circumstances and recommendation for discussion

The case describes changed circumstances requiring an appropriate discussion.

2. What was Quinn's preference?

D. Avoid intervention if possible

Quinn hoped to avoid intervention and wants that preference acknowledged.

3. What is not supplied?

D. A procedure selection or fetal tracing

The exercise does not provide the clinical evidence or a selected intervention.

D Language checks / page 45

1. Which statement preserves involvement?

D. Your preferences still matter as we explain the changed recommendation.

The patient remains involved even when circumstances require a new clinical discussion.

2. What is not equivalent to consent?

C. Understanding the explanation

A person can understand a recommendation without agreeing to it.

3. What should time sensitivity describe?

A. The actual urgency and available decision process

The timing discussion must reflect the real circumstances rather than pressure or unsupported evidence.

4. Which clinical action is not authorized here?

B. Selecting the birth intervention

No procedure or clinical decision is supplied for the learner to select.

05 Conversation answers

Revisiting a birth preference

E Dialogue gaps

Pages 46-47

- 1. birth preference.** Turn 2. Birth preferences remain relevant when changed circumstances require a new discussion.
- 2. clinical indication.** Turn 4. A clinical indication explains the actual reason supporting a proposed intervention.
- 3. time sensitivity.** Turn 6. Time sensitivity describes real urgency, not a tactic for preventing questions.
- 4. assisted vaginal birth.** Turn 8. Assisted vaginal birth names an option; mentioning it does not establish consent.
- 5. procedural benefit.** Turn 10. Procedural benefit is an expected advantage, not a guaranteed outcome.
- 6. reasonable alternative.** Turn 12. A reasonable alternative is a relevant option requiring explanation in context.
- 7. informed consent.** Turn 14. Informed consent requires appropriate authorization after a meaningful discussion and is not established merely by a nod or comprehension of words.
- 8. analgesia.** Turn 16. Analgesia concerns pain relief and should be discussed through the actual clinical arrangements rather than assumed from the procedure name.
- 9. consent discussion.** Turn 18. A consent discussion addresses the proposed care, relevant information, questions, and authorization rather than treating a signature as the whole process.
- 10. shared decision-making.** Turn 20. Shared decision-making combines the actual clinical information with the person's informed preferences and questions, including when circumstances change.

F Transfer conversation

Page 48

1. heard

Patient: "My nod meant I heard the explanation."

The patient clarifies that the nod acknowledged hearing the explanation.

2. consent

Clinician: "It did not establish consent."

Hearing an explanation does not automatically authorize the proposed intervention.

3. question

Patient: "I still have a question."

The scenario continues the discussion rather than treating authorization as complete.

4. process

Clinician: "We will continue the appropriate process."

The actual consent process continues with the patient's clarification acknowledged.

05

06 Briefing and language checks

Giving a labor-ward handoff

A Briefing checks / page 49

1. What does Samira report?

D. Stronger contractions

The case supplies the patient's report of stronger contractions only.

2. What preference should be handed over?

C. Explain before examinations

Samira asks for explanations before examinations, which remains relevant to the next team.

3. What requires verification?

C. Current observations and monitoring interpretation

The actual clinical values and interpretation must come from the current record.

D Language checks / page 54

1. Which statement is source-aware?

C. Samira reports stronger contractions.

The statement accurately attributes the supplied information to the patient.

2. What does handoff acceptance add?

D. Confirmation of received information and responsibility

Acceptance closes the communication loop rather than merely recording that a message was sent.

3. Which finding cannot be invented?

A. A fetal trace classification

No trace or classification is supplied in the case.

4. Why include the examination preference?

B. It remains relevant to communication and consent.

The preference guides the next team's explanation and does not disappear during handoff.

06 Conversation answers

Giving a labor-ward handoff

E Dialogue gaps

Pages 55-56

1. contraction pattern. Turn 3. Contraction pattern needs source attribution; patient report is not a measured finding.

2. maternal observations. Turn 5. Maternal observations require actual clinical findings and the relevant assessment time.

3. cervical dilation. Turn 7. Cervical dilation cannot be inferred from a report of stronger contractions.

4. Membrane status. Turn 9. Membrane status needs its actual source and current assessment context.

5. fetal heart-rate trace. Turn 11. A fetal heart-rate trace requires clinical interpretation, not inference from symptoms.

6. Variability. Turn 13. Variability describes fluctuations in fetal heart rate within the monitoring context and its definition does not provide a clinical interpretation algorithm.

7. outstanding task. Turn 15. An outstanding task is an action not yet completed and needs an explicit owner rather than an assumption that the next team knows.

8. handoff acceptance. Turn 17. Handoff acceptance confirms receipt of the information and responsibility, making the transfer more than an unacknowledged message.

9. contingency plan. Turn 19. A contingency plan specifies responses under the actual care plan and cannot be invented from an incomplete language scenario.

10. working assessment. Turn 20. A working assessment is the current clinical interpretation subject to review and must remain separate from unverified values or assumed conclusions.

F Transfer conversation

Page 57

1. 10:00

Receiver: "That observation was recorded at 10:00."

The supplied observation belongs to the earlier 10:00 assessment.

2. 12:00

Sender: "The handoff is occurring at 12:00."

The current handoff time is 12:00 in the scenario.

3. current

Receiver: "We need the actual current record."

The earlier value must not be presented as a current finding.

4. timestamp

Sender: "I will correct the timestamp."

The immediate communication error concerns the observation's time attribution.

06

07 Briefing and language checks

Taking postpartum concerns seriously

A Briefing checks / page 58

1. When did Sloane give birth?

B. Twelve days ago

The brief places the birth twelve days before this urgent concern.

2. Which symptoms are reported?

B. Shortness of breath and chest pain

Sloane reports breathing difficulty and chest pain without an established cause.

3. What is already active?

D. The emergency response

The actual emergency response is active and must not be delayed.

D Language checks / page 63

1. Which detail belongs near the start of the handoff?

B. Birth twelve days ago with current chest pain and breathlessness

The current symptoms and recent birth context are the supplied urgent communication priorities.

2. Which conclusion is unsupported?

D. The cause is postpartum cardiomyopathy.

The case supplies no diagnostic evidence establishing a particular cause.

3. What does a routine appointment not replace?

A. Actual urgent assessment

A future routine review is not a substitute for the active urgent clinical process.

4. Which mental-health distinction is important?

C. An intrusive thought is not automatically an intention to act.

Thought content, distress, intention, and actual risk require appropriate assessment rather than automatic equivalence.

07 Conversation answers

Taking postpartum concerns seriously

E Dialogue gaps

Pages 64-65

1. recent birth history. Turn 2. Recent birth history provides context that belongs alongside the current symptoms.

2. postpartum warning sign. Turn 4. A postpartum warning sign must not be dismissed as ordinary tiredness.

3. shortness of breath. Turn 6. Shortness of breath is a symptom; the patient need not diagnose its cause.

4. chest pain. Turn 8. Chest pain requires assessment and does not identify its cause by itself.

5. postpartum period. Turn 10. The postpartum period identifies the time after childbirth and provides context without independently establishing the cause of a new symptom.

6. thromboembolism. Turn 12. Thromboembolism is a clinical condition requiring appropriate assessment and is not established by appearing in a list of possible concerns.

7. Postpartum cardiomyopathy. Turn 14. Postpartum cardiomyopathy is a heart-muscle disorder requiring clinical evaluation and is not confirmed by the reported symptoms alone.

8. Postpartum follow-up. Turn 16. Postpartum follow-up is planned care after birth and does not replace urgent assessment of a new concerning symptom.

9. handover context. Turn 18. Handover context includes relevant background such as recent birth and must accompany the current symptom report to the receiving team.

10. continuity of care. Turn 20. Continuity of care connects information and responsibility across services and time, including after an urgent episode.

F Transfer conversation

Page 66

1. twelve

Sender: "The omitted detail is birth twelve days ago."

The scenario supplies a birth twelve days before the current concern.

2. history

Receiver: "I acknowledge the added history."

The receiving team confirms receipt of the recent birth information.

3. active

Sender: "The response remains active."

Adding the missing context does not interrupt the ongoing response.

4. established

Receiver: "The cause is not yet established."

The added birth history does not independently establish a diagnosis.

08 Briefing and language checks

Discussing confirmed pregnancy loss

A Briefing checks / page 67

1. What has been confirmed?

B. An early pregnancy loss after appropriate assessment

The finding is confirmed, while cause and management choice remain separate questions.

2. What worries Eden?

C. That an ordinary activity caused the loss

Eden expresses self-blame about an ordinary daily activity.

3. What is not prescribed?

C. A management option or follow-up interval

The case arranges an individualized discussion rather than prescribing treatment or timing.

D Language checks / page 72

1. Which statement matches the case?

D. The loss is confirmed, but the cause is not established.

The case separates the confirmed clinical finding from the unresolved cause.

2. Which phrase minimizes the loss?

B. At least it was early.

At least it was early imposes a minimizing comparison rather than acknowledging the person's experience.

3. What does empathetic silence allow?

B. Space for the person to respond

A pause allows a response without rushing the person toward reassurance or a decision.

4. Which action is not supplied?

D. Selecting a management regimen

The scenario supplies no treatment selection, medicine regimen, or clinical follow-up interval.

08 Conversation answers

Discussing confirmed pregnancy loss

E Dialogue gaps

Pages 73-74

- 1. confirmed finding.** Turn 2. The finding confirms loss; it does not establish why the loss occurred.
- 2. self-blame.** Turn 4. Self-blame needs acknowledgment without inventing evidence that an ordinary activity caused loss.
- 3. Acknowledgment of loss.** Turn 6. Acknowledgment of loss recognizes its significance rather than minimizing the experience.
- 4. Empathetic silence.** Turn 8. Empathetic silence allows a response without rushing toward reassurance or decisions.
- 5. support preference.** Turn 10. Support preference identifies whom the patient wants involved in the discussion.
- 6. expectant management.** Turn 12. Expectant management involves observation under a clinical plan and is an option term, not an instruction here to wait without assessment.
- 7. Medical management.** Turn 14. Medical management uses prescribed medicines under the actual clinical plan; the term itself does not specify a regimen or establish suitability.
- 8. management preference.** Turn 16. A management preference is the person's informed choice among relevant options and should be elicited after the actual clinical information is explained.
- 9. bereavement support.** Turn 18. Bereavement support offers help after a significant loss and should be presented as available support rather than an obligation or a substitute for clinical care.
- 10. care summary.** Turn 20. A care summary records the confirmed finding, relevant discussion, and agreed next steps without presenting unresolved choices as already decided.

F Transfer conversation

Page 75

1. finding

Patient: "You may share the confirmed finding."

Eden authorizes sharing the established finding with her sister.

2. sister

Clinician: "The support person is your sister."

The person joining the discussion is Eden's chosen sister.

3. selected

Patient: "A management option is not yet selected."

No management choice has been made in the supplied scenario.

4. separate

Clinician: "I will keep those facts separate."

Permission to share a finding does not establish a treatment decision.

08

09 Conversation answers

A sensitive consultation with language support

E Extended conversation

Pages 76-77

1. qualified language support. Turn 2. Qualified language support provides trained interpretation for accurate participation and is distinct from assuming a companion should translate sensitive information.

2. disclosure permission. Turn 4. Disclosure permission concerns authorization to share specified information and does not follow automatically from a companion's language ability.

3. language preference. Turn 8. Language preference identifies the language or variety the patient understands best and should be checked rather than inferred from nationality or a companion.

4. clarification request. Turn 12. A clarification request asks for an unclear message to be explained and supports informed participation rather than being treated as unreasonable delay.

5. decision-making role. Turn 16. The patient's decision-making role remains distinct from the interpreter's communication role and must not be transferred merely because language support is needed.

6. communication agreement. Turn 20. A communication agreement records the relevant preferences and arrangements for language, privacy, support, and contact without replacing the actual clinical consent process.

F Transfer exchange

Page 78

1. shared

Patient: "The listed number is shared."

The listed telephone is used by the patient and a companion.

2. route

Clinician: "You have requested a different route."

The patient asks for another way to receive communication.

3. verified

Patient: "That route is not yet verified."

The service has not yet confirmed that the requested route is available and appropriate.

4. information

Clinician: "We will check before sending sensitive information."

The contact arrangement must be verified before sensitive material is sent.

10 Conversation answers

Discussing postpartum contraception preferences

E Extended conversation

Pages 79-80

1. reproductive preferences. Turn 4.

Reproductive preferences describe the person's wishes about future pregnancy and should be elicited without pressure to commit to a fixed timetable.

2. Reversibility. Turn 6. Reversibility concerns whether and how a method's contraceptive effect can be ended and needs explanation for the actual option being discussed.

3. bleeding concern. Turn 8. A bleeding concern identifies an effect important to the patient's choice and should be discussed using the actual method information rather than dismissed.

4. Medical eligibility. Turn 10. Medical eligibility assesses whether a method is appropriate for the person's actual clinical circumstances and cannot be reduced to a universal rule from one detail.

5. voluntary choice. Turn 14. Voluntary choice belongs to the informed patient and should not be replaced by pressure from family or a clinician's preference.

6. preference discussion. Turn 18. A preference discussion explores priorities and information needs and is not itself consent to a procedure or a prescription.

F Transfer exchange

Page 81

1. Reversibility

Patient: "Reversibility matters to me."

The patient identifies reversibility as an important preference.

2. bleeding

Clinician: "You also asked about bleeding changes."

Possible bleeding changes are the second stated information need.

3. selected

Patient: "A method has not been selected."

The discussion has not reached a method choice.

4. prescribed

Clinician: "Nothing has been prescribed in this scenario."

No prescription is supplied or authorized by this language exercise.

11 Conversation answers

An uncertain ultrasound finding

E Extended conversation

Pages 82-83

1. uncertain finding. Turn 2. An uncertain finding is an observation whose significance is not yet established and should not be converted into a confirmed diagnosis.

2. specialist review. Turn 6. A specialist review examines the relevant finding and context with appropriate expertise and does not itself predetermine the conclusion.

3. source images. Turn 10. Source images are the original examination images needed for review and are distinct from a shortened description copied into another document.

4. measurement limitation. Turn 12. A measurement limitation affects what a particular value can establish and requires the actual examination context rather than an unrelated comparison.

5. further-test decision. Turn 18. A further-test decision requires the actual information and appropriate informed discussion rather than being inferred from a request for clarification.

6. Result follow-up. Turn 20. Result follow-up assigns the route and responsibility for explaining the reviewed findings rather than ending the process when a referral is sent.

F Transfer exchange

Page 84

1. missing

Patient: "The source images are missing."

The specialist service has not yet received the original examination images.

2. received

Clinician: "The referral letter has been received."

Receipt is confirmed for the letter, not the complete review material.

3. confirmed

Patient: "The appointment is not confirmed."

No actual scheduled appointment has been confirmed in the scenario.

4. uncertain

Clinician: "The finding remains uncertain."

The incomplete transfer does not establish a diagnostic conclusion.

Find the move you need

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

01 Taking a sensitive pregnancy history

Ask permission: May we discuss your previous pregnancy experiences?

Ask about words: Is there a term you would prefer us to use?

Summarize respectfully: Let me check that I have understood your history correctly.

Full phrase set: pages 7-8 | Dialogue: pages 10-11

02 Explaining prenatal screening choices

Name the result type: This is a screening result, not a diagnosis.

Offer counseling: We can arrange a discussion of the actual result and options.

Allow a pause: We can pause to make sure the information is clear.

Full phrase set: pages 16-17 | Dialogue: pages 19-20

03 Escalating an urgent maternal concern

Lead with symptoms: You report a severe headache and changes in vision.

Preserve unknowns: We do not yet have a verified value to report.

Prevent delay: Do not wait for this conversation to finish before following the urgent process.

Full phrase set: pages 25-26 | Dialogue: pages 28-29

04 Explaining a glucose-testing pathway

Separate the stages: We need to distinguish this screening result from a diagnostic conclusion.

Clarify preparation: We will verify the instructions for the actual test.

Carry follow-up forward: Let us identify who will coordinate the next result discussion.

Full phrase set: pages 34-35 | Dialogue: pages 37-38

Keep the conversation productive

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

05 Revisiting a birth preference

Acknowledge the preference: I remember that avoiding intervention was important to you.

Make time clear: I will explain how time-sensitive the decision is.

Keep the patient involved: Your questions and preferences remain part of this discussion.

Full phrase set: pages 43-44 | Dialogue: pages 46-47

06 Giving a labor-ward handoff

Lead with context: I am handing over Samira on the labor ward.

Verify the record: Let us check the current observations and their times.

Close the transfer: I acknowledge the handoff and the unresolved items.

Full phrase set: pages 52-53 | Dialogue: pages 55-56

07 Taking postpartum concerns seriously

State recent pregnancy: I gave birth twelve days ago.

Avoid dismissal: We will not assume this is ordinary recovery.

Close the loop: We will confirm that the receiving team has the relevant history.

Full phrase set: pages 61-62 | Dialogue: pages 64-65

08 Discussing confirmed pregnancy loss

Give clear news: I am sorry. The assessment has confirmed that the pregnancy has ended.

Respond to blame: Nothing in this assessment establishes that your ordinary activity caused this.

Close concretely: We will provide the agreed care plan and the appropriate contact route.

Full phrase set: pages 70-71 | Dialogue: pages 73-74

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

acknowledgment of loss	offer acknowledgment of loss	69
amniocentesis	discuss amniocentesis	14
analgesia	discuss analgesia options	41
anesthesia	explain anesthesia arrangements	41
assisted vaginal birth	discuss assisted vaginal birth	41
baseline rate	describe the baseline rate	50
bereavement support	offer bereavement support	69
birth preference	acknowledge a birth preference	41
bleeding history	obtain a bleeding history	68
blood-pressure measurement	relay a blood-pressure measurement	23
capillary glucose	discuss capillary glucose	32
care summary	provide a care summary	69
cell-free DNA screening	explain cell-free DNA screening	14
cervical dilation	document cervical dilation	50
cervical effacement	describe cervical effacement	50
cesarean birth	explain cesarean birth	41
change in circumstances	describe a change in circumstances	42
chest pain	report chest pain	59
chorionic villus sampling	explain chorionic villus sampling	14
chromosomal condition	discuss a chromosomal condition	15
clinical handoff	confirm the clinical handoff	6
clinical handoff	confirm the clinical handoff	15
clinical handoff	confirm the clinical handoff	24
clinical handoff	confirm the clinical handoff	33

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

clinical handoff confirm the clinical handoff	42
clinical handoff confirm the clinical handoff	51
clinical handoff confirm the clinical handoff	60
clinical handoff confirm the clinical handoff	69
clinical indication explain the clinical indication	41
clinical observation distinguish clinical observations	24
communication support arrange communication support	42
confirmed finding explain a confirmed finding	68
consent discussion document the consent discussion	42
contingency plan clarify the contingency plan	51
continuity of care maintain continuity of care	60
contraction pattern report the contraction pattern	50
dating ultrasound discuss a dating ultrasound	5
dating uncertainty document dating uncertainty	6
deceleration describe a deceleration	50
decision capacity assess decision capacity	41
decision support offer decision support	15
diagnostic testing discuss diagnostic testing	14
diagnostic threshold verify diagnostic thresholds	32
diagnostic uncertainty distinguish diagnostic uncertainty	68
documented plan review the documented plan	6
documented plan review the documented plan	15
documented plan review the documented plan	24
documented plan review the documented plan	33

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

documented plan review the documented plan	42
documented plan review the documented plan	51
documented plan review the documented plan	60
documented plan review the documented plan	69
early pregnancy loss explain early pregnancy loss	68
ectopic pregnancy clarify an ectopic-pregnancy history	5
empathetic silence allow empathetic silence	69
epigastric pain clarify reported epigastric pain	23
escalation pathway activate the escalation pathway	24
estimated due date verify the estimated due date	5
expectant management discuss expectant management	68
false-negative result explain a false-negative result	14
false-positive result explain a false-positive result	14
fasting sample clarify fasting-sample instructions	32
fetal assessment coordinate fetal assessment	24
fetal heart-rate trace review the fetal heart-rate trace	50
fetal station report fetal station	50
follow-up assessment arrange follow-up assessment	68
follow-up ownership confirm follow-up ownership	33
functional change describe functional change	60
genetic counseling arrange genetic counseling	14
gestational age explain gestational age	5
gestational diabetes discuss gestational diabetes	32
glucose log review a glucose log	32

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

glucose screening	explain glucose screening	32
glucose tolerance test	discuss a glucose tolerance test	32
gravity	document gravity	5
handoff acceptance	confirm handoff acceptance	51
handover context	preserve handover context	60
history clarification	request history clarification	6
hyperglycemia	discuss hyperglycemia	33
hypertensive disorder	assess a hypertensive disorder	23
hypoglycemia	explain hypoglycemia	33
inconclusive result	communicate an inconclusive result	15
individualized target	explain individualized targets	32
informed consent	obtain informed consent	41
informed refusal	document informed refusal	15
insulin resistance	explain insulin resistance	32
intrusive thought	clarify an intrusive thought	59
labor progress	describe labor progress	50
last menstrual period	clarify the last menstrual period	5
live birth	document a live birth	5
management preference	elicit a management preference	68
material risk	discuss material risks	41
maternal observations	relay maternal observations	51
maternal status	communicate maternal status	24
maternal warning sign	communicate a maternal warning sign	23
maternity triage	contact maternity triage	23

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

meal context document meal context	33
medical management explain medical management	68
membrane status clarify membrane status	50
menstrual regularity ask about menstrual regularity	5
miscarriage discuss a miscarriage	68
monitoring schedule confirm the monitoring schedule	33
nutrition consultation arrange a nutrition consultation	32
obstetric history obtain an obstetric history	5
outstanding task identify outstanding tasks	51
parity clarify parity	5
patient preference elicit the patient preference	6
patient preference elicit the patient preference	15
patient preference elicit the patient preference	24
patient preference elicit the patient preference	33
patient preference elicit the patient preference	42
patient preference elicit the patient preference	51
patient preference elicit the patient preference	60
patient preference elicit the patient preference	69
patient question invite patient questions	42
perinatal mental health discuss perinatal mental health	59
positive predictive value discuss positive predictive value	14
postpartum cardiomyopathy evaluate postpartum cardiomyopathy	59
postpartum follow-up coordinate postpartum follow-up	60
postpartum hemorrhage assess postpartum hemorrhage	59

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

postpartum hypertension	evaluate postpartum hypertension	59
postpartum period	identify the postpartum period	59
postpartum testing	arrange postpartum testing	33
postpartum warning sign	report a postpartum warning sign	59
postprandial	explain postprandial measurements	32
preeclampsia	evaluate suspected preeclampsia	23
preferred terminology	ask about preferred terminology	6
prenatal record	review the prenatal record	6
prenatal screening	explain prenatal screening	14
presenting part	identify the presenting part	50
preterm birth	review a preterm-birth history	5
prior pregnancy loss	acknowledge a prior pregnancy loss	5
procedural benefit	explain procedural benefits	41
proteinuria	interpret proteinuria	23
qualified interpreter	request a qualified interpreter	6
qualified interpreter	request a qualified interpreter	15
qualified interpreter	request a qualified interpreter	24
qualified interpreter	request a qualified interpreter	33
qualified interpreter	request a qualified interpreter	42
qualified interpreter	request a qualified interpreter	51
qualified interpreter	request a qualified interpreter	60
qualified interpreter	request a qualified interpreter	69
reasonable alternative	explain reasonable alternatives	41
receiving service	confirm the receiving service	24

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

recent birth history communicate recent birth history	59
residual risk explain residual risk	14
retained pregnancy tissue assess retained pregnancy tissue	68
right-upper-quadrant pain describe right-upper-quadrant pain	23
rupture time verify rupture time	50
screen-positive result explain a screen-positive result	14
screening probability communicate screening probability	14
self-blame respond to self-blame	69
severe headache report a severe headache	23
shared decision-making support shared decision-making	42
shortness of breath describe shortness of breath	59
source attribution preserve source attribution	6
source attribution preserve source attribution	15
source attribution preserve source attribution	24
source attribution preserve source attribution	33
source attribution preserve source attribution	42
source attribution preserve source attribution	51
source attribution preserve source attribution	60
source attribution preserve source attribution	69
suicidal thought ask directly about suicidal thoughts	59
support network identify the support network	60
support person identify the chosen support person	6
support preference ask about support preferences	69
surgical management discuss surgical management	68

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

symptom progression report symptom progression	23
teach-back use teach-back without blame	6
teach-back use teach-back without blame	15
teach-back use teach-back without blame	24
teach-back use teach-back without blame	33
teach-back use teach-back without blame	42
teach-back use teach-back without blame	51
teach-back use teach-back without blame	60
teach-back use teach-back without blame	69
test limitation explain test limitations	15
testing pathway clarify the testing pathway	32
thromboembolism assess suspected thromboembolism	59
time sensitivity explain time sensitivity	41
trauma-informed approach use a trauma-informed approach	6
unconfirmed diagnosis preserve an unconfirmed diagnosis	24
urgent assessment arrange urgent assessment	23
urgent-care route confirm the urgent-care route	60
uterine activity assess uterine activity	51
variability assess variability	50
viability assessment review the viability assessment	68
visual disturbance clarify a visual disturbance	23
voluntary testing support voluntary testing	15
withdrawal of consent clarify withdrawal of consent	42
working assessment state the working assessment	51

Original cases. Clear boundaries.

These are original fictional teaching conversations, not transcripts of real workers. Names, figures, organizations, and incidents are invented. Sources support terminology and communication aims, not the fictional results or an endorsement of this book.

[1] ACOG. Prenatal Genetic Screening Tests.

[Open the original source](#)

Background for distinguishing screening probabilities from diagnostic conclusions and discussing testing choices. Checked 10 October 2026.

[2] ACOG. Gestational Diabetes.

[Open the original source](#)

Terminology and communication context; no diagnostic cutoffs or treatment instructions are supplied. Checked 10 October 2026.

[3] CDC Hear Her. Urgent Maternal Warning Signs.

[Open the original source](#)

Supports prompt attention to urgent concerns during pregnancy and after birth; the language tasks never determine that symptoms can wait. Checked 10 October 2026.

[4] ACOG. Early Pregnancy Loss.

[Open the original source](#)

Context for compassionate discussion of confirmed loss, uncertainty, and individualized options without prescribing management. Checked 10 October 2026.

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